DIGITALAIKART

Premium AI digital products, made for India

120 COPY-PASTE PROMPTS · 10 PARTS · 7 APPENDICES

The AI Prompt Pack for Indian Dietitians & Nutritionists

Client onboarding · Indian meal plans · GLP-1 support · WhatsApp retention · handouts · content · pricing · records · troubleshooting

2026-27 EDITION

For registered dietitians, clinical nutritionists and qualified nutrition coaches in India

This is a working tool for a qualified professional. It is not clinical guidance.

FAISAL FAYAZ LONE

About this pack

This is a working tool, not a book. It contains **120 copy-paste prompts** across ten parts of a dietitian's week, plus seven appendices of reference material you will use more often than the prompts themselves. Every prompt was written for the Indian consulting room: dal and roti, joint families, hostel mess food, festival weeks, WhatsApp follow-up, and the new GLP-1 client who arrived in Indian clinics in 2026.

It assumes one thing: that a qualified professional is reading the output before a client does. Nothing in this pack is clinical guidance, and no prompt in it is a substitute for your training, your judgement, or a doctor's opinion.

A note on accuracy

Market data, drug prices, brand names and search trends change quickly in India. Every external figure in this pack is attributed to the reporting that published it and dated to 2026. Where a number may have moved, the pack says so and tells you to re-check. Where an AI assistant is likely to invent a figure, the prompt is written to delete the number rather than guess at it.

Edition and terms

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F	The 90-Day Implementation Plan
G	The One-Page Cheat Sheet

How to Use This Pack

Start with the Master Context block

Paste the block below into a fresh chat once, before you use any prompt in this pack. It tells the assistant who you are, who your clients are, and what it must never do. Keep that chat open and add prompts to it through the day - the assistant remembers the frame, and you stop repeating yourself.

Fill the brackets, then edit

Every prompt has square brackets. Fill them with real, specific detail. The difference between a useless output and a usable one is almost always the specificity of what you typed in the brackets. Then edit the result in your own voice before it reaches a client - a client can tell when a handout was not written by their dietitian.

Use the 'Use when' line

Each prompt has one. It tells you the moment the prompt earns its place. If you cannot match it to a moment in your week, skip it - this pack is a working tool, not a reading book.

Keep the review discipline

Nothing leaves your desk without the pre-send review in Part 9, Prompt 114. Two minutes there is the difference between a helpful document and a problem.

Run the safety rails below, every time

They are not legal decoration. They are the reason a dietitian can use AI and an influencer cannot.

The Master Context block

Paste this once, at the start of a new chat. It is the single highest-value thing in this pack.

You are assisting me, a dietitian and nutritionist practising in India. I am the qualified professional; you are my drafting assistant.

ABOUT MY PRACTICE
I work with: [your client types - e.g. adults with PCOS, insulin resistance, fatty liver, weight management, sports nutrition]
Mode: [online / clinic / both] Location: [city, state]
Client languages: [English, Hindi, and any others]
My approach: [describe your method in 3-5 sentences - this is the most important line in this block]

HOW I WANT YOU TO WRITE
- Simple Indian English. Short sentences. No jargon without a definition.
- Use Indian food names and household measures: katori, bowl, glass, chapati, ladle. Never grams unless I ask.
- Never call a food "bad", "junk" or a "cheat". Never use body-shaming language.
- No motivational filler. No "you've got this". Warm, but plain.

WHAT YOU MUST NOT DO
- Do not diagnose, prescribe, dose, or advise on medication.
- Do not give calorie, gram or macro figures unless I explicitly ask and supply the method.
- Do not invent statistics, studies, prices or product names. Write "not established" or "verify" instead.
- Do not promise weight loss, reversal of a condition, or any specific outcome.
- Do not ask me for client names, phone numbers, addresses or report identifiers. If I include them by mistake, tell me to remove them.

WHEN A CLIENT HAS A MEDICAL CONDITION
Give me the supportive nutrition frame only, and list separately what must go to their doctor. One consolidated caution at the end of your answer, not a hedge in every sentence.

END EVERY CLIENT-FACING DRAFT WITH
"This is a draft for review by a qualified professional. It is not medical advice."

Confirm you have understood by replying with one line, and wait for my first task.

The Seven Rules

These are not legal decoration. They are the reason a qualified dietitian can use AI in client work while an Instagram coach cannot do the same thing safely.

01 AI drafts, you decide

The assistant writes the first draft. You are the professional whose name is on it. Never send an unread output to a client, a group or a public page.

02 No numbers you did not set

Models estimate Indian portions badly. Ask for dishes, not grams. Set every quantity yourself, from your own judgement and your client's data.

03 No diagnosis, ever

Not in a handout, not in a caption, not in a group reply. If a symptom needs a diagnosis, it needs a doctor. Say so and move on.

04 One caution, not a hedge in every line

Put the medical caveat where it belongs - once, at the end - and write the rest of the document confidently. Hedging everything makes the advice useless.

05 Strip the identity before you paste

No names, no phone numbers, no addresses, no report identifiers, no photos with faces. Age band, sex, city and the clinical picture are enough for the AI to be useful.

06 Verify every external fact

Prices, drug brands, scheme rules and market figures change constantly in India. Anything current is a starting point, not a source of truth. Check it before it reaches a client.

07 The client's own food wins

A plan built on dal, roti, rice, sabzi, idli, poha and curd gets followed. A plan built on oats and salads gets abandoned in four days. Force the model back to the Indian kitchen every time.

PART 1 OF 10

Client Onboarding & Discovery

The first two conversations decide whether a client stays for three months or disappears after week two. These prompts turn a messy intake into a clean brief you can work from in five minutes.

12 PROMPTS IN THIS PART

01 The Master Client Brief
02 The WhatsApp intake form, short version
03 Reading a three-day food diary
04 The 'I have tried everything' client
05 Cooking for a joint family
06 The food budget reality check
07 The red-flag triage
08 The 20-minute first consult agenda
09 Process goals, not kilos
10 The expectation-setting conversation
11 The 24-hour recap message
12 The first reply to an enquiry

1 The Master Client Brief

USE WHEN *Every new client, before your first call.*

You are helping me prepare for a nutrition consultation. I am a dietitian in India. Below is what the client told me, in their own words. Do not diagnose, do not prescribe, do not calculate any calorie or gram figure.

CLIENT (anonymised): [age band, sex, city, occupation, family situation]
WHAT THEY SAID: [paste their WhatsApp messages or form answers, with names removed]
WHAT THEY WANT: [their goal, in their words]

Give me:
1. A four-line clinical-style summary of the situation, in neutral language.
2. The three things they said that actually matter most for behaviour change.
3. What they did NOT tell me that I still need to ask (max 6 questions).
4. Any red flag that means I should refer them to a doctor before I plan anything.
5. One sentence I can open the call with that shows I listened.

Keep it under 300 words. Use plain English, no jargon.

Note *The value here is question 4. A referral you make in the first call protects the client and your practice. Never let the model answer question 4 with a diagnosis - it should only name what needs checking.*

2 The WhatsApp intake form, short version

USE WHEN *When a lead asks 'what do you need from me?'*

Write a 10-question intake form for a new nutrition client in India, to be sent on WhatsApp as a single message they can reply to in one go.

Rules:
- Maximum 10 questions, numbered, each answerable in one line.
- Cover: age, height and weight (if they know), what they eat in a normal day, cooking situation (self / family / cook / mess / hostel / takeaway), food they will not give up, medical conditions and medicines they are on, sleep, work hours, and what they have already tried.
- Do not ask for a full medical history - that comes on the call.
- Warm, respectful, no body-shaming language. Do not use the words 'obese', 'fat' or 'cheat meal'.
- End with one line telling them what happens next and how long it takes.
- Under 180 words total, so it fits one screen.

Note *Short forms get filled. Long forms get ignored - that is the whole reason this exists. The 'food they will not give up' question is the one that makes the plan stick.*

3 Reading a three-day food diary

USE WHEN *Before a follow-up, when the client sends a diary or photos.*

```
Here is a client's three-day food record, written in their own words. I am a dietitian; you are my assistant reading it for patterns, not judging health.

RECORD: [paste the diary, anonymised]

Do not estimate calories or grams. Instead give me:
1. Eating pattern: how many eating occasions, gaps longer than 5 hours, skipped meals, and the time of day most intake happens.
2. Protein: which meals had a clear protein source, and which had none. Use Indian food names.
3. Fibre and vegetables: where they appear and where they are missing.
4. The one repetitive habit that is easiest to change first.
5. Three things they are already doing well that I should name out loud in the session.
6. Anything in the record I should ask a doctor about.

Bullet points only, under 250 words, supportive tone.
```

Note *Ask for 'what they are doing well' every single time. Clients arrive carrying guilt about food; a session that opens with praise is the one they come back to.*

4 The 'I have tried everything' client

USE WHEN *The client on their fourth diet plan, low on trust.*

```
A client has told me: 'I have tried everything, nothing works for me.' I am a dietitian in India.

Their context: [age band, what they have tried, how each attempt ended, family situation, work hours]

Give me:
1. A 60-second spoken response that validates the frustration without agreeing that nothing works.
2. Three questions that uncover why the earlier attempts failed - built around their life, not their willpower.
3. A reframe that moves the conversation from 'a diet to follow' to 'a system that survives a bad week'.
4. One small commitment I can ask for in this session that is almost impossible to fail.
5. The one sentence I must avoid saying, because it will sound like every other person who has lectured them.

Write it as things I would actually say out loud, in simple Indian English.
```

Note *Point 5 is the point of the prompt. Ask the model to name the cliché you would otherwise fall into - usually 'you just need discipline'.*

5 Cooking for a joint family

USE WHEN *When the client does not control the kitchen.*

My client eats what the family cooks. They do not decide the menu, cannot ask for separate food, and do not want to offend anyone at home.

Situation: [who cooks, how many people eat, typical dishes, who buys groceries, any family comments about their weight, region and cuisine]

Give me:
1. Five changes that can be made to the family's existing dishes without cooking anything separate - additions, swaps, or serving-order changes.
2. How to phrase the request to the person who cooks so it sounds like a small favour, not a criticism of their food.
3. Three things the client can control entirely at the table, whatever is served.
4. One line the client can use when a relative says 'eat, you have become thin' or 'diet means you are unwell'.
5. What I should tell the client NOT to do, because it will cause conflict at home.

Keep it practical, warm, and specific to Indian home cooking.

Note *Most Indian diet failures are kitchen-politics failures, not knowledge failures. Solving the household is more effective than solving the menu.*

6 The food budget reality check

USE WHEN *Before writing any plan for a client on a tight budget.*

My client has a limited monthly food budget. I am a dietitian in India.

BUDGET: about Rs [amount] per month for food, for [number] people. City: [city]. Cooking: [gas / induction / shared kitchen]. Equipment available: [pressure cooker, mixer, fridge?]

Give me:
1. The cheapest reliable protein sources available in an Indian kirana or sabzi mandi, ranked by protein per rupee - and say clearly which of your figures are estimates I must check locally.
2. Five meals built only from those ingredients.
3. Where a client on this budget usually overspends without noticing.
4. What to buy in bulk versus fresh, and roughly how long each keeps.
5. What NOT to cut, even on a tight budget, because removing it makes the plan fail.

Do not invent exact prices. Where you are unsure, write 'check local price'.

Note *The 'do not invent prices' instruction is essential. Model prices for Indian groceries are frequently wrong and a wrong number destroys your credibility in one message.*

7 The red-flag triage

USE WHEN *Before you accept any client - run this every time.*

I am a dietitian in India, deciding whether I can take this client or must refer them first. You are not diagnosing. You are giving me a checklist to think against.

WHAT THE CLIENT TOLD ME: [paste, anonymised]

List, in two columns:
COLUMN A - situations where I must insist on a doctor's clearance before I give any diet advice.
COLUMN B - situations I can work with, but must flag and monitor.
Then: for each item you list, give me one line I can say to the client that refers them without losing them as a client.

Be conservative. If you are unsure whether something is a red flag, put it in Column A. Add a closing note: 'This is a checklist for professional judgement, not a diagnosis.'

Note *Keep this prompt saved as a favourite. It is the single most protective prompt in the pack - for the client and for you.*

8 The 20-minute first consult agenda

USE WHEN *The day before every first consultation.*

Build me a 20-minute agenda for a first nutrition consultation with this client.

CLIENT: [one-line profile: age band, main goal, main constraint]
MY SETUP: [phone call / video / in-clinic]

For each block give the minutes, the one question that opens it, and what I must have written down by the end of it. Include:
- a 90-second opening that sets expectations,
- the part where I collect what the intake form missed,
- the part where I say what I can and cannot do (scope, referral, no guarantees),
- the part where I ask for the food record and the follow-up slot,
- a 60-second close that tells them exactly what happens next and when.

Total must be 20 minutes. Then give me a 5-line template for what I send them within 24 hours.

Note *A stated agenda cuts no-shows and makes the paid hour feel professional. The 'what I cannot do' block prevents the most common client complaint later.*

9 Process goals, not kilos

USE WHEN *When a client's only goal is a number on the scale.*

My client's goal is: [e.g. lose 10 kg before a wedding in 3 months]. I am a dietitian in India.

Give me:
1. A gentle way to explain that I cannot promise a number, and what I can promise instead - without sounding like I am dodging.
2. Four process goals we can actually track weekly, specific to an Indian household (not 'eat healthy').
3. A simple weekly scorecard the client can fill on WhatsApp in under a minute - four rows maximum.
4. What to say if they hit the goal weight early and want to stop.
5. What to say if the scale has not moved in three weeks but clothes fit better.

Plain English, no motivational filler, no promises of results.

Note *Process goals are what you can defend. Never let a document - or a model - put a guaranteed kg figure in front of a client.*

10 The expectation-setting conversation

USE WHEN *Before you take money, and again at the first review.*

Write the expectation-setting part of my first consultation, as words I would say out loud.

I am a dietitian in India. I want to cover, in under three minutes and without frightening the client:
- what my role is, and what it is not,
- that I do not diagnose or change medicines,
- that results depend on their week, not only on my plan,
- that if something medical shows up I will send them to a doctor,
- how often we will speak and how they reach me between sessions,
- what happens if they miss two check-ins.

Then give me the same six points as a short WhatsApp message the client can keep and re-read. Warm, plain, no legal language.

Note *Say it once, in writing, and you never have to argue about it at month end. The WhatsApp version becomes your onboarding message.*

11 The 24-hour recap message

USE WHEN *Within a day of every consultation.*

Turn my rough notes from a consultation into a client recap message.

MY NOTES: [paste your scribbles]
CLIENT: [first name only]
WHAT WE AGREED: [the two or three changes]

Write a WhatsApp message that:
- confirms the two or three changes in the client's own words where possible,
- names one thing they are already doing well,
- states the next check-in date and what they need to send before it,
- ends with one warm line, no emoji spam (maximum one emoji),
- is under 150 words and readable on a phone.

Do not add advice we did not agree on. Do not add any new food rules.

Note *Recaps increase compliance more than any plan document. The instruction 'do not add advice we did not agree on' keeps the model from quietly inventing a fourth rule.*

12 The first reply to an enquiry

USE WHEN *The 60 seconds after someone messages you on Instagram or WhatsApp.*

Someone has messaged me asking about a diet consultation. I am a dietitian in India.

WHAT THEY SAID: [paste the message]
WHAT I KNOW ABOUT THEM: [whatever is visible - city, age if stated, what they asked]

Write my first reply so that:
1. It answers the question they actually asked, in the first two lines.
2. It asks one question - the one that qualifies them fastest - and no more than one.
3. It tells them what happens next, in a single sentence.
4. It does not send a price list before I know anything about them.
5. It does not promise a result, and does not say 'DM for details'.

Under 80 words. Warm, human, no emoji spam. Then give me the follow-up I send if they reply, and the one I send if they go quiet for 48 hours.

Note *One question, not five. Enquiries die when the first reply reads like a form.*

PART 2 OF 10

Indian Meal-Plan Drafting

The core of the work - and the part AI is worst at on its own, because it reaches for oats, salads and quinoa. These prompts force it to build on dal, roti, rice, sabzi, curd, idli, poha and the food your client actually eats.

16 PROMPTS IN THIS PART

01 The Master Meal-Plan Frame
02 North Indian vegetarian week
03 South Indian week
04 Bengali and Odia week
05 Gujarati and Maharashtrian thali week
06 Jain, satvik and no-onion-garlic plans
07 The high-protein vegetarian fix
08 The Rs 150-a-day plan
09 The office dabba plan
10 The hostel and mess-food plan
11 The shift-worker plan
12 Festival and fasting weeks
13 Wedding season and eating out
14 The grocery list builder
15 Portions without a weighing scale
16 The client who hates vegetables

1 The Master Meal-Plan Frame

USE WHEN *Set this up once. Use it for every plan you draft.*

You are drafting a meal plan skeleton for me. I am a dietitian in India and the clinical decisions are mine. Your job is to lay my method over this client's week, not to bring a diet of your own.

MY METHOD: [describe your principles in 5-7 sentences - how you balance meals, meal timing, what you always include, what you never do]

CLIENT: [age band, sex, goal, activity level, city, region and cuisine, vegetarian/egg/non-veg, foods they refuse, allergies, cooking time available, budget, who cooks]

Build a 7-day skeleton: breakfast, mid-morning, lunch, evening, dinner. For each slot give 2 interchangeable options so the client has choice. Repeat base dishes across the week so the grocery list is realistic. Use Indian home food with Indian names.

Rules:
- No calorie, gram or macro numbers anywhere. I will set portions myself.
- Flag any dish that is unsuitable for a stated allergy in capitals.
- Write 'I cannot judge this' if a stated medical condition makes a dish unsafe, rather than guessing.
- End with a list of what I must check before sending this to the client.

Note *This one prompt is worth the price of the pack. The 'no numbers' rule is deliberate - models estimate Indian portions badly, and a wrong number in a client's plan is your name on it.*

2 North Indian vegetarian week

USE WHEN *Punjabi, UP, Delhi, Haryana, Rajasthan clients.*

Draft a 7-day North Indian vegetarian meal skeleton.

CLIENT: [goal, age band, activity, foods refused, cooking time, budget]

Build it on what this kitchen already cooks: roti or paratha, dal, seasonal sabzi, curd, rice, chaach, occasional paneer. Include a chaat or street-food-style evening option once, because a plan with no joy does not survive. Keep breakfast options to things made in under 15 minutes on a weekday - poha, upma, besan chilla, stuffed paratha with curd, eggs if they eat them.

For each day give: breakfast (2 options), lunch, evening, dinner. Show where the protein is in each meal, using Indian food names.

No calorie or gram figures. Note in one line what makes this plan specifically North Indian rather than generic Indian.

Note *Naming the region forces specificity. Without it you get a plan that reads like a Delhi client and a Chennai client could swap.*

3 South Indian week

USE WHEN *Tamil Nadu, Karnataka, Andhra, Telangana, Kerala clients.*

Draft a 7-day South Indian meal skeleton.

CLIENT: [goal, age band, activity, foods refused, cooking time, state]

Build it on idli, dosa, upma, pongal, sambar, rasam, poriyal, kootu, curd rice, avial, fish curry where relevant. Respect the state - Kerala coconut and fish, Andhra spice and pickles, Tamil Nadu sambar and rasam, Karnataka ragi and akki rotti.

Include: 2 breakfast options per day that are realistic for a working household, lunch, evening, dinner. Mark the protein source in each meal and name at least three cheap protein additions that are common in South Indian kitchens.

No calorie or gram figures. Flag anything that is high in salt or oil, because that is the usual hidden problem in this cuisine.

Note *The salt-and-oil flag matters. South Indian home food is often nutritionally strong and sodium-heavy, and clients rarely see the second part.*

4 Bengali and Odia week

USE WHEN *Clients whose staple is rice and fish.*

Draft a 7-day Bengali or Odia meal skeleton.

CLIENT: [goal, age band, activity, foods refused, cooking time, whether they eat fish, egg, chicken]

Build it on rice, dal, machher jhol, shaak, chorchori, begun bhaja, doi, and for Odia clients dalma, saga, santula, chhena. Keep the meal structure this kitchen actually uses - rice at lunch, lighter at night - and do not replace rice with a foreign grain.

For each day: breakfast, lunch, evening, dinner. Mark the protein in each meal. Name three ways to raise protein without removing rice.

No calorie or gram figures. One line on the biggest nutritional trap in this cuisine.

Note *Rice is not the enemy; the missing protein and the frying oil usually are. This prompt keeps the model from doing the lazy thing of cutting rice.*

5 Gujarati and Maharashtrian thali week

USE WHEN *Clients eating a thali pattern - the most balanced format in India.*

Draft a 7-day Gujarati or Maharashtrian thali-style meal skeleton.

CLIENT: [goal, age band, activity, foods refused, sweet tolerance, cooking time, state]

Build it on the thali logic the household already uses: roti or bhakri, dal or kadhi, shaak or bhaji, rice, curd, salad, and one small sweet or farsan where the family expects it. For Maharashtra include bhakri, thalipeeth, usal, misal, pithla, varan bhaat. For Gujarat include dhokla, khichdi, kadhi, undhiyu in season.

For each day: breakfast, lunch, evening, dinner. Show where protein sits and how to lift it without adding ghee or sugar. Keep the one small sweet if the family will not accept its removal - and say how to fit it in.

No calorie or gram figures.

Note *Keeping the sweet is a deliberate design choice. Plans that delete the thing a family eats daily get abandoned in ten days.*

6 Jain, satvik and no-onion-garlic plans

USE WHEN *Jain clients, fasting households, temple-adjacent families.*

Draft a 7-day vegetarian meal skeleton that is Jain-friendly.

Hard constraints: no onion, no garlic, no potato, no root vegetables, no egg, no fish, no meat, and nothing cooked the previous evening if the client follows that practice. Dairy is allowed unless stated otherwise.

CLIENT: [goal, age band, activity, region, cooking time, foods refused]

For each day: breakfast, lunch, evening, dinner - two options per slot. Use dishes that are genuinely Jain in an Indian home, not vegetarian dishes with the onion deleted. Mark the protein source in every meal and tell me the three biggest protein gaps this constraint creates, with one fix for each.

No calorie or gram figures. If a constraint makes a balanced week very hard, say so plainly instead of pretending it is easy.

Note *Jain clients are poorly served by generic plans, and there are a lot of them. The honest closing line is what makes the output usable.*

7 The high-protein vegetarian fix

USE WHEN *The single most common gap in Indian plans.*

My vegetarian client needs more protein, and I do not want to sell them powder.

CLIENT: [goal, age band, activity, region, foods refused, budget, whether they eat paneer, curd, soya, sprouts, eggs]

Give me:
1. Twelve vegetarian protein sources available in a normal Indian kitchen, grouped by how easy they are to add to an existing meal.
2. For each, one line on how much of it is a realistic portion and one dish that uses it - in Indian names.
3. Five specific additions to meals they already eat, not new meals.
4. The three mistakes that make vegetarian Indian diets low in protein even when dal is eaten daily.
5. What to say to a client who believes dal alone is enough.

Do not give gram targets - I will set those. Do not invent absorption or digestibility statistics.

Note *The 'do not invent statistics' line saves you. Ask for dishes, not numbers - dishes you can check, numbers you cannot.*

8 The Rs 150-a-day plan

USE WHEN *Students, single earners, clients in shared accommodation.*

Draft a 7-day meal skeleton for a client whose total food spend is about Rs [amount] per day, cooking for one.

CITY: [city]. KITCHEN: [what they have]. DIET: [veg / egg / non-veg]. GOAL: [goal].

Rules:
- Only ingredients available in a neighbourhood kirana or sabzi mandi.
- Nothing that needs an oven, a blender, or more than 20 minutes.
- Reuse ingredients across the week so nothing spoils.
- For each day: breakfast, lunch, evening, dinner, with two options where cheap.
- Mark protein in every meal.
- Add a shopping list grouped by shop type, with a note on what to buy weekly versus daily.

No prices in rupees - you will get them wrong. Tell me instead which five items dominate the spend so I can check them locally.

Note *Asking for 'what dominates the spend' instead of prices is how you get something usable from a model that cannot price Indian groceries.*

9 The office dabba plan

USE WHEN *Clients who carry tiffin and eat one meal out.*

Build a weekday tiffin plan for a client who carries lunch to an Indian office.

CLIENT: [goal, age band, commute time, whether there is a fridge and microwave at work, who packs the tiffin, diet type]

Give me:
1. Five tiffin combinations that survive four hours without a fridge and do not leak or smell strong in a shared cabin.
2. What to pack separately so it stays fresh.
3. Three breakfasts that take under 10 minutes for someone leaving at [time].
4. What to do about the 4 pm office snack and the office tea or coffee run.
5. The two days a week when eating out at the office canteen is unavoidable - what to order.

No calorie figures. Keep it to food a normal Indian household cooks.

Note *Logistics, not nutrition, is why tiffin plans fail. The fridge and smell details are the ones clients actually care about.*

10 The hostel and mess-food plan

USE WHEN *Students and working professionals with no kitchen at all.*

My client lives in a hostel or PG and eats only mess or canteen food. They cannot cook.

SITUATION: [city, mess timings, what the mess usually serves, what they can buy, budget for extras, diet type, goal]

Give me:
1. A realistic day built from what a typical Indian hostel mess serves - breakfast, lunch, evening, dinner.
2. Five things they can keep in their room without a fridge, and five that need only hot water.
3. How to build a plate at a mess counter when the dal is watery and the sabzi is oily.
4. Three cheap additions from a campus shop that fix the protein gap.
5. What to do on the days the mess food is fried or unappetising.
6. One habit to drop that is costing them the most.

No calorie figures. Be honest that this is a compromise, not an ideal - do not pretend mess food can be perfect.

Note *Honesty converts better than an ideal plan. A client who is told 'this is a compromise' trusts the rest of the advice more.*

11 The shift-worker plan

USE WHEN *Nurses, IT support, factory, BPO, drivers, doctors.*

Build an eating plan for a client on a rotating or night shift.

SHIFT: [pattern, e.g. 7 nights on, 3 off, or 2 pm to 11 pm]
CLIENT: [goal, age band, diet type, who cooks, sleep pattern, caffeine intake]

Give me:
1. A 24-hour eating map for the shift, showing when to eat the main meal and when to keep it light - and why.
2. What to eat at 2 am that does not sit heavy, using Indian food.
3. How to handle the transition day when the shift changes.
4. Three ways to manage tea and coffee without a crash.
5. What to do about the family meal they do not want to miss.
6. The one thing that matters more than the food plan for this client.

No calorie figures. Note where this interacts with medication timing and tell me to send them to their doctor for that.

Note *The medication-timing note is not optional. Shift work plus diabetes or thyroid medication is a doctor's call, not a plan's.*

12 Festival and fasting weeks

USE WHEN *Navratri, Karva Chauth, Ramzan, Ekadashi, Ganesh Chaturthi, Diwali week.*

Build a plan for a client during [festival or fasting period], which lasts [number] days.

CLIENT: [goal, diet type, region, whether they fast fully or partially, what they traditionally eat, whether they have diabetes]

Give me:
1. What to eat on a fasting day, in the order it should be eaten, using the foods their household actually prepares.
2. How to handle the sweets and fried food that arrive as gifts, without a rule that nobody follows.
3. The one risk I must warn them about, given their health situation.
4. What to do on the day after the festival.
5. A one-line message they can send family who insist they eat more.

Then tell me plainly which part of this I should check with their doctor before sending.

Note *Festival advice that says 'avoid all sweets' is ignored. The 'order it should be eaten' framing is what actually changes intake.*

13 Wedding season and eating out

USE WHEN *November to February, and any client with a heavy social calendar.*

My client has [number] weddings, parties and dinners in the next six weeks and does not want to opt out of any of them.

CLIENT: [goal, diet type, region, how much they drink, what they usually overeat at events]

Give me:
1. A pre-event eating rule for the day of the function, so they arrive not starving.
2. What to put on the plate at a buffet, in order, for North Indian and for South Indian menus.
3. How to handle the dessert counter and the drinks, without a total ban.
4. The next-day recovery day - not a punishment day.
5. Two lines they can say when relatives push food.
6. What to tell them about the scale for those six weeks.

No calorie figures, no moralising about food.

Note *The 'not a punishment day' instruction is what stops the binge-restrict cycle that ruins the whole season.*

14 The grocery list builder

USE WHEN *Attach to every plan you send.*

Here is a 7-day meal plan I have finalised. Turn it into a shopping list.

PLAN: [paste the final plan]
HOUSEHOLD: [number of people eating, city, whether they shop daily or weekly]

Give me:
1. A consolidated list with duplicates removed, grouped as: vegetables and fruit, dals and pulses, dairy, grains and flours, oils and spices, other.
2. Which items to buy weekly and which to buy fresh every two days.
3. A 'check at home first' list - the spices and staples most kitchens already have.
4. Any ingredient that is hard to find outside a big city, with a common substitute.
5. What will spoil first, and the order to cook things in.

No quantities in grams and no rupee prices. Use household units - packets, bunches, katoris, litres.

Note *Clients who have a clear list follow the plan. This is the cheapest compliance win in the whole pack.*

15 Portions without a weighing scale

USE WHEN *Every client who will never buy a kitchen scale - which is most of them.*

Teach my client to judge portions without weighing anything.

CLIENT: [goal, age band, diet type, region, literacy level, whether they cook]

Give me:
1. A hand-and-katori guide using things every Indian kitchen has - katori, bowl, glass, chapati, ladle, and the client's own palm and fist.
2. A sample plate for lunch and for dinner, described in those units, for their goal.
3. Three visual checks they can do at a restaurant where nothing is measured.
4. How to explain that portions matter less than what is on the plate, without confusing them.
5. A one-page version they can screenshot and keep.

No gram weights anywhere. This must be usable by someone who has never weighed food.

Note *Gram-based plans are followed for four days. Katori-based plans are followed for four months.*

16 The client who hates vegetables

USE WHEN *More common than anyone admits, and never solved by saying 'eat more vegetables'.*

My client eats almost no vegetables and will not start eating salads.

CLIENT: [age band, what vegetables they will eat, what they refuse and why - texture, taste, childhood, family habit, region]

Give me:
1. Ten ways to get vegetables into Indian food they already eat, without the client noticing a 'vegetable dish' - grated, pureed into gravy, added to dal, hidden in paratha, mixed into rice.
2. The five vegetables that are easiest to introduce to someone who dislikes them, and how to cook each so the texture changes.
3. What to do about the bitterness problem specifically.
4. Three dishes that are genuinely vegetable-forward but do not look or taste like a 'healthy' dish.
5. What NOT to do - the approaches that make a fussy eater dig in harder.
6. What to tell the family member who is frustrated with them.

No calorie figures. No 'just try it' advice.

Note *Texture, not taste, is usually the real objection. Point 2 is where the plan either works or does not.*

PART 3 OF 10

GLP-1 and Metabolic Support

India's weight-loss drug market changed completely in 2026. Semaglutide lost patent protection on 20 March 2026, generic versions arrived at a fraction of the innovator price, and a new kind of client walked into clinics: eating far less, losing fast, and needing nutrition support nobody had planned for. This part is built for that client.

14 PROMPTS IN THIS PART

01 The GLP-1 client intake add-on
02 Protein-first for a reduced appetite
03 The nausea, constipation and reflux day plan
04 The 'should I take it?' boundary
05 The off-medicine transition plan
06 Prediabetes: the 12-week frame
07 Fatty liver (MASLD) food frame
08 PCOS and insulin resistance
09 The thyroid client
10 The plateau at week eight
11 The family-worried conversation
12 The doctor coordination note
13 The metabolic risk explainer
14 The muscle-protection brief

1 The GLP-1 client intake add-on

USE WHEN *Any client who mentions Ozempic, Wegovy, Mounjaro, or a generic semaglutide.*

A client has told me they are on a GLP-1 medicine. I am a dietitian in India. I do not prescribe, adjust or comment on the dose - that is their doctor's job.

WHAT THEY TOLD ME: [what they said, anonymised - brand if they know it, how long, who prescribed it]

Give me:
1. The questions I must ask that are within my scope - appetite, meal size, nausea, bowel habits, protein intake, strength training, sleep, alcohol, and what they are eating now.
2. The questions I must NOT ask, because they belong to the prescriber.
3. The three things I should flag back to their doctor, and how to word that note professionally.
4. What a client on this medicine most often gets wrong about food in the first month.
5. One line I can say that makes clear I support them without endorsing or opposing the medicine.

No dosing advice. No opinion on whether they should be on it.

Note *The 'questions I must not ask' list is the scope line that keeps you safe. Keep this prompt and reuse it for every GLP-1 client.*

2 Protein-first for a reduced appetite

USE WHEN *The central nutrition problem on a GLP-1: they cannot eat enough to protect muscle.*

My client is eating very little because their appetite has dropped sharply. Their goal is to protect muscle while losing fat.

CLIENT: [age band, diet type, what they can tolerate, how many meals they manage, region, cooking situation]

Give me:
1. A day structured so protein comes first in every eating occasion, using Indian food that is easy on a small appetite.
2. Ten small-volume, protein-dense Indian options - things that fit in a katori, not a thali.
3. What to do when they can only finish half a meal.
4. Three soft or liquid protein options for the days solid food feels like too much.
5. The signs that they are losing muscle rather than fat, and what I should do about it.
6. What I must refer to a doctor.

No gram targets - I will set those with the client. No supplement brand recommendations.

Note *Muscle loss is the real risk in rapid weight loss, and it is the part clients never ask about. Point 5 is the value of this prompt.*

3 The nausea, constipation and reflux day plan

USE WHEN *The three most common side effects clients bring to a dietitian.*

My client is managing [nausea / constipation / reflux / a combination] while eating much less than before.

CLIENT: [which symptom, how long, what they currently eat, diet type, region, any other medicine]

Give me:
1. A one-day eating plan for the symptom, using Indian food that is gentle and familiar - not toast and crackers.
2. Foods and cooking methods to lean on, and ones to pause for now.
3. Practical timing changes, not just food changes.
4. The red-flag signs that mean they must see their doctor rather than adjust food.
5. A short message they can send their prescriber describing the symptom accurately.

Say clearly that this is supportive nutrition advice and not treatment for the side effect.

Note *A client describing symptoms accurately to their prescriber is a real clinical contribution you can make as a dietitian. Point 5 earns you referrals.*

4 The 'should I take it?' boundary

USE WHEN *The question every dietitian in India is being asked in 2026.*

A client has asked me whether they should start a weight-loss injection.

CONTEXT: [their situation, what they have tried, their health conditions, who else is advising them]

Write my response, as words I would say, that:
1. Answers honestly that this is a decision for them and their doctor, and says why - not as a dodge but as a real reason.
2. Names what I can do either way: the food and muscle-protection work that matters on or off the medicine.
3. Asks the three questions that will help them have a better conversation with their doctor.
4. Does not shame them for asking, and does not pretend the medicine is a shortcut.
5. Offers one concrete next step in my own practice.

Warm, plain Indian English, under 220 words. Do not recommend for or against any brand or molecule.

Note *This prompt protects you commercially too. The dietitian who answers this well becomes the person the client calls after the prescription.*

5 The off-medicine transition plan

USE WHEN *Clients stopping or tapering, or who cannot afford to continue.*

My client is stopping or reducing their GLP-1 medicine. Appetite will come back and they are worried.

CONTEXT: [how long they were on it, how much they lost, what they eat now, activity, what they can afford, family situation]

Give me:
1. What usually happens to appetite and weight after stopping, in plain language I can share.
2. A four-week food structure for the transition, built on Indian home food, that rebuilds normal meal size gradually.
3. The habits from the medicine period worth keeping - and the ones that must change.
4. How to handle the first month of returning hunger without panic.
5. What to tell them about the scale in weeks 4 to 12.
6. What I should refer to their doctor.

No predictions of exact weight regain. Be honest that this varies.

Note *This is the underserved conversation in the whole GLP-1 wave. Most content online is about starting; almost none is about stopping.*

6 Prediabetes: the 12-week frame

USE WHEN *The largest addressable client group in India.*

Build a 12-week nutrition structure for a client with prediabetes. I am a dietitian in India; the diagnosis and monitoring are their doctor's.

CLIENT: [age band, sex, city, diet type, family history, work hours, activity, what they eat now, whether they have a glucometer]

Give me:
1. Week-by-week focus, one change added per week, never more.
2. For each week: the single change, why it matters, and how the client will know it is working without a lab test.
3. The three Indian food habits that drive post-meal glucose spikes most, with the specific swap for each.
4. How to structure a South Indian and a North Indian meal so the carbohydrate is not eaten alone.
5. What to say about rice - honestly.
6. The exact point at which I must send them back to their doctor.

No calorie figures, no promises of reversal, no supplement advice.

Note *One change a week is the difference between a programme and a pamphlet. Never promise reversal - promise process.*

7 Fatty liver (MASLD) food frame

USE WHEN *Increasingly common, and rarely explained well to Indian clients.*

My client has been told they have fatty liver. I am a dietitian in India.

CLIENT: [age band, weight trend, alcohol use, diet type, region, activity, other conditions, medicines]

Give me:
1. A plain-language explanation of what fatty liver means, in under 150 words, that does not frighten them and does not minimise it.
2. The food changes with the strongest evidence behind them - and say plainly which parts are well established and which are still debated.
3. A 7-day Indian meal skeleton that fits those changes, with two options per slot.
4. What to do about sugar, jaggery, honey and fruit juice - the four things Indian clients most often get wrong.
5. What I must hand back to their doctor, including anything about alcohol.
6. The three questions they should ask their doctor at the next visit.

No claims of cure. No numbers I cannot verify.

Note *Jaggery and honey are the classic Indian blind spot - clients believe 'natural' means 'safe'. Point 4 is the most valuable page in this prompt.*

8 PCOS and insulin resistance

USE WHEN *One of the highest-volume client groups in Indian dietetics.*

Build a nutrition approach for a client with PCOS and suspected insulin resistance. Diagnosis and medication are their doctor's.

CLIENT: [age, city, diet type, cycle pattern as they describe it, activity, sleep, stress, what they have tried, whether they are on any medicine]

Give me:
1. What to explain to the client about the link between insulin resistance and PCOS, in under 200 plain words.
2. A 7-day Indian meal skeleton built for steady energy - breakfasts that are not sweet, and why.
3. The five swaps that matter most in an Indian kitchen.
4. How to talk about weight without making it the only outcome we measure.
5. What to do about cravings in the week before a period.
6. The three things I must not promise.
7. What to refer to their gynaecologist or endocrinologist.

No calorie figures. No claims about fertility outcomes.

Note *Fertility claims are the fastest way to lose your licence and your reputation. Point 6 exists for that reason.*

9 The thyroid client

USE WHEN *Very common, heavily mythologised.*

My client has hypothyroidism and is on medication. I am a dietitian in India and will not touch their dose.

CLIENT: [age, weight trend, diet type, region, other conditions, what they have read online, medicines they take]

Give me:
1. What food can and cannot do here - stated plainly, without hype.
2. A 7-day Indian meal skeleton suited to their goal.
3. The five most common thyroid myths Indian clients repeat, each with a one-line, respectful correction.
4. The timing questions I must send to their doctor - tablets, food, calcium, iron, coffee.
5. What to say to a client who has stopped their medicine and wants food to replace it.
6. The signs that mean they need a review, not a new plan.

No supplement advice. No promises about weight loss speed.

Note *Point 5 is a real safety issue. Rehearse that answer before you need it.*

10 The plateau at week eight

USE WHEN *The moment most programmes are lost.*

My client has lost weight steadily and then stopped. They are at week [number] and frustrated.

WHAT HAS CHANGED: [what they report eating now versus at the start, activity, sleep, stress, whether they are measuring anything]
WHAT HAS NOT CHANGED: [what they insist is the same]

Give me:
1. The five most likely explanations, ranked, given what they have told me - and for each, the one question that would confirm or rule it out.
2. What I should NOT assume (for example, that they are lying or that they need to eat less).
3. Three changes to try, in order, before touching quantity at all.
4. What to say to a client who wants to cut food drastically.
5. The honest conversation about what a plateau is, in under 100 words.
6. When to send them for a medical review.

No calorie maths. No 'reverse diet' jargon unless you explain it in one line.

Note *Point 2 matters more than the rest. Most plateaus are adherence drift, sleep, or measurement error - not a metabolic emergency.*

11 The family-worried conversation

USE WHEN *When a spouse or parent is pushing back on the plan or the medicine.*

```
My client's family is worried or critical about what they are doing - [the specific objection, e.g. 'you are not eating rice', 'these injections are dangerous', 'just eat ghee and stop this diet'].

CONTEXT: [who is objecting, client's age and situation, what the client wants]

Give me:
1. A short, respectful explanation the client can give at the dinner table - three sentences maximum.
2. What NOT to say, because it will turn a meal into an argument.
3. One concession the client can offer that costs nothing nutritionally but buys peace at home.
4. What I can say if the family member speaks to me directly.
5. A line that works with an older relative who equates thinness with illness.

No lecturing, no statistics, no confrontation.
```

Note *Family objection is the number one reason Indian clients quit a plan. Buying peace cheaply is a legitimate clinical strategy.*

12 The doctor coordination note

USE WHEN *Whenever a client is under medical care for the same issue.*

```
Write a short professional note from me, a dietitian, to the client's doctor.

CLIENT: [age band, condition, what I am doing nutritionally, what the client reported]
WHAT I NEED FROM THE DOCTOR: [a specific question, or 'nothing - informational only']

The note should:
- be under 180 words, formal but not stiff,
- state my role and that I am not advising on medication,
- summarise what the client reported, in neutral language,
- state what nutrition plan I have set, briefly,
- ask the specific question, if there is one,
- close with my contact details placeholders.

Do not include any diagnosis of my own. Do not include the client's identifying details beyond what a doctor needs.
```

Note *One well-written note per client turns a doctor from a stranger into a referral source. Send it as a PDF on letterhead, not a WhatsApp paragraph.*

13 The metabolic risk explainer

USE WHEN *For the client who does not understand why any of this matters.*

Explain to my client, in simple Indian English, why their [condition] matters for the long term - without frightening them and without a lecture.

CLIENT: [age, condition, what they already know, education level, language comfort]

Give me:
1. A 120-word explanation using one everyday comparison - not a medical analogy.
2. Three things that are in their control, and one that is not.
3. What changes in the first month if they do nothing, versus if they start now - described in life terms, not lab values.
4. The single sentence they should remember.
5. A short version they can send to a family member who is not taking it seriously.

No mortality statistics. No scare tactics. No numbers you cannot verify.

Note *Fear produces one good week and then avoidance. The 'one thing that is not in their control' line is what makes the rest believable.*

14 The muscle-protection brief

USE WHEN *For any client losing weight fast, and every GLP-1 client.*

My client is losing weight quickly and I want to protect their muscle. I am a dietitian; exercise prescription is not my scope, so this is a brief for their trainer or for their own doctor.

CLIENT: [age band, how fast they are losing, activity now, any joint or mobility limitation, what they can access - gym, home, park]

Give me:
1. A one-page brief I can hand to a trainer, stating what the nutrition side is doing and what I need the training side to cover.
2. Three questions I should ask the client about strength work without prescribing it.
3. The signs, in a client's own description, that muscle is being lost rather than fat.
4. What to say to a client who insists they will only walk.
5. The protein-timing conversation, in plain language, without gram targets.
6. Where this must go to their doctor instead.

No exercise programmes, no rep schemes, no supplement advice.

Note *Handing a trainer a one-page brief is how dietitians get repeat referrals. Prescribing the training yourself is how they stop.*

PART 4 OF 10

WhatsApp, Follow-Up and Retention

In Indian dietetics the plan is 20% of the outcome and the follow-up is 80%. Retention, not acquisition, is the biggest moat in this market - clients start engaged and drift by week four. These prompts keep them in the programme.

14 PROMPTS IN THIS PART

01 The photo food-log protocol
02 The weekly check-in message
03 The three-day silence nudge
04 The 'I cheated' reply
05 The festival-week proactive message
06 The mid-programme review summary
07 The renewal conversation
08 The price objection reply
09 The early-exit reply
10 The referral ask
11 The testimonial with consent
12 The conflicting-advice reply
13 The supplement push-back
14 The 'no results' honest conversation

1 The photo food-log protocol

USE WHEN *Set this up in week one, for every client.*

Write the instructions I give a client for logging food by photograph on WhatsApp, for [number] weeks.

CLIENT: [age band, work pattern, phone type, whether they are comfortable sending photos]

The message should cover:
1. What to photograph - the plate before eating, not the empty plate.
2. How to make it useful in one second - angle, lighting, and a hand or spoon for scale.
3. What to write in the caption - three words maximum.
4. How often, and what to do on days they forget.
5. What I will do with the photos and when they get feedback.
6. A reassurance that this is data, not judgement, and that nothing is graded.

Under 200 words, warm, one emoji maximum, readable on a phone.

Note *Photo logging lifts compliance far above written diaries because it costs the client three seconds. The 'nothing is graded' line is what keeps them sending.*

2 The weekly check-in message

USE WHEN *Every week, same day, same time. Build the habit.*

Write my standard weekly check-in message to a client on a nutrition programme.

CLIENT: [first name], week [number], goal [goal]
WHAT WE ARE TRACKING: [the two or three things]

The message should:
- open with something specific to them, not a template greeting,
- ask for exactly three pieces of information, numbered so they can reply 1-2-3,
- remind them of the one focus for this week,
- tell them when they will hear back from me,
- be under 120 words.

Then give me a shorter 40-word version for clients who reply with one word, and a version for a client who has gone quiet.

Note *Numbered questions get answered. Paragraph questions get a thumbs-up emoji and nothing else.*

3 The three-day silence nudge

USE WHEN *When a client stops replying.*

A client has not replied for three days. Their context: [week number, what they were struggling with, how engaged they were before].

Write three messages I can send over the next week:
1. Day 3 - light, no guilt, one easy ask.
2. Day 5 - names the likely obstacle and offers to make the plan easier.
3. Day 8 - a clear, respectful note about what happens to their programme if we do not speak.

Each under 70 words. No shaming, no 'I noticed you have been quiet' opener, no threats. The third message must be honest about the consequences without being cold.

Note *Most 'lost' clients are overwhelmed, not uninterested. The second message - offering to make the plan easier - recovers more clients than any discount.*

4 The 'I cheated' reply

USE WHEN *Every single week, from someone.*

A client has messaged me saying they 'cheated' or 'spoiled everything' after eating [what they ate, how much, at what occasion].

Write my reply so that:
1. I remove the word 'cheat' from the conversation without lecturing them about language.
2. I normalise it in one line that is true, not a platitude.
3. I ask one question that gets useful information - not a confession.
4. I give one adjustment for the next 24 hours, small and specific.
5. I close by restating the one focus for the week.

Under 110 words, warm, no exclamation marks, no emoji. Then give me a version for a client who does this every week and needs a different conversation.

Note *Shame is the mechanism that turns one bad meal into one bad month. This is the highest-leverage 100 words in your whole practice.*

5 The festival-week proactive message

USE WHEN *Send it before the festival, not after.*

Write a proactive message I send all my clients the week before [festival], in [year].

AUDIENCE: my client list, mixed goals and diet types, mostly [city or region]

The message should:
1. Acknowledge the festival warmly and specifically - name two foods associated with it.
2. Give three practical rules for the week that do not ban anything.
3. Give one rule for the day after.
4. Tell them what NOT to do (weighing daily, skipping meals to 'save' for the evening).
5. Offer a slot for anyone who wants to talk before the week starts.

Under 200 words, warm, no moralising, maximum one emoji. Then give me a 30-word version for a WhatsApp status or broadcast.

Note *A message sent before the festival prevents the week of silence after it. This one message saves more programmes than any plan revision.*

6 The mid-programme review summary

USE WHEN *Week 4, 8 and 12 of every programme.*

Turn my notes from a review session into a written progress summary for the client.

MY NOTES: [what changed, what they reported, what the measurements show, what we decided]
PROGRAMME: week [number] of [total], goal [goal]

Give me:
1. A short 'what has changed' section, written in plain language, that names non-scale wins first.
2. A 'what we are keeping' section - the habits that are working.
3. A 'what we are changing' section - maximum two things.
4. The next four weeks in one line.
5. A closing paragraph that is honest about pace.

Under 300 words. No numbers I did not give you. No new advice.

Note *The written review is what clients show their family when someone asks why they are paying you. It is also what they re-read in a bad week.*

7 The renewal conversation

USE WHEN *Two weeks before a package ends.*

```
My client's [number]-week programme ends in two weeks. I want to talk about continuing, without pressure.

WHAT WE ACHIEVED: [specific wins]
WHAT IS STILL OPEN: [what is unfinished]
NEXT STAGE OPTIONS: [the two packages I can offer]

Give me:
1. The opening line that does not sound like a sales call.
2. How to present what is unfinished as an opportunity, honestly, not as a failure.
3. A one-paragraph description of each option, in outcome language not feature language.
4. The price line, said plainly, with no apology.
5. What to say if they say 'let me think about it' - and what to say if they say no.
6. What to send them the day after, whichever way it goes.

No urgency tactics, no fake deadlines, no discounts invented by you.
```

Note *'Let me think about it' is usually a pricing question in disguise. Point 5 is where most of the revenue is.*

8 The price objection reply

USE WHEN *The most common objection in Indian dietetics.*

```
A prospective client has said my fee of Rs [amount] per month is too expensive, because [their reason - a cheaper option, a free app, a family member's opinion].

MY OFFER: [what is included, how often we speak, what support they get]
THEIR SITUATION: [what they told me]

Write my reply so that:
1. I do not immediately discount - I first make the offer legible by breaking it into what it costs per day.
2. I name what the cheaper alternative does not include, without criticising it.
3. I offer one smaller entry point if it genuinely exists, and I do not invent one.
4. I am comfortable letting them walk away.
5. The reply is under 130 words and can be sent on WhatsApp.

Then tell me the one thing I should stop saying in these conversations.
```

Note *Breaking a monthly fee into a per-day figure is the single most effective, honest reframe in a price conversation. Do the arithmetic yourself before sending.*

9 The early-exit reply

USE WHEN *When a client wants to stop mid-programme or asks for a refund.*

A client wants to stop [number] weeks into a [number]-week programme, or has asked for a refund. Their reason: [reason].

MY POLICY: [your actual policy - be accurate]

Write my reply so that:
1. I acknowledge the reason without arguing with it.
2. I state the policy plainly and once, without justifying it three times.
3. I offer the one thing I can genuinely offer - a pause, a lighter plan, or a call.
4. I leave the door open in a way that is sincere, not a tactic.
5. I ask for one piece of feedback, in a single question.

Under 150 words. No guilt, no mention of how much effort I put in.

Note *Never let AI invent a refund policy. Paste your real one in. The 'state it once' instruction is what stops the defensive over-explaining.*

10 The referral ask

USE WHEN *The week a client has a visible win.*

My client has just had a good result - [what happened]. I want to ask for a referral without making it awkward.

Write three versions of the ask:
1. A direct one for a client I know well.
2. A soft one for a client who is private.
3. A version that gives them something to forward - a short line they can send to a friend who asked about their progress.

Each under 80 words. No pressure, no rewards or incentives invented by you, no 'just share my number with ten friends'.

Note *The forwardable line is the highest-converting part. Most referrals die because the client does not know what to say.*

11 The testimonial with consent

USE WHEN *Only after the win, and only with permission.*

I want to ask a client for a testimonial, and I need to do it properly.

CLIENT: [what they achieved, how long they worked with me, their comfort with being public]

Give me:
1. The request message, which asks permission first and explains exactly how it will be used.
2. Three prompts that help them write it themselves, in their own words.
3. A consent line covering name or initials, city, photo use, and the right to withdraw.
4. What I must never publish without a separate, explicit yes.
5. A version for a client who wants to stay anonymous.

Under 200 words total. Plain language, no legal jargon beyond what is needed.

Note *Health testimonials are personal data. Get the consent line right once and use it every time - it is also what makes the testimonial credible.*

12 The conflicting-advice reply

USE WHEN *'My friend lost 12 kg on keto.' 'A reel said rice is poison.'*

A client has come to me with advice from someone else: [the specific claim and where it came from].

CLIENT: [their situation, whether they are tempted, what they have tried]

Give me:
1. A response that respects the source and the client, and still gives a clear position.
2. The one honest thing that is true in the other advice - because there usually is one.
3. Why it may not work for this client specifically, in one concrete sentence.
4. What to say when the client wants to try it anyway.
5. A line for the client to use with the person who gave the advice.

Under 200 words. No mocking the other approach, no 'studies show' without a real study.

Note *Finding the true part of the other advice is what makes you credible. Clients can tell the difference between a fair answer and a defensive one.*

13 The supplement push-back

USE WHEN *When a client arrives with an influencer's supplement stack.*

My client wants to buy supplements recommended by [source]: [list what they were told to take].

CLIENT: [their diet, any conditions, medicines, blood work they have, budget]

Give me:
1. A neutral, factual line for each item - what it is claimed to do, and what is genuinely established about it, clearly separating the two.
2. Which of these I must send to their doctor before they spend money.
3. The three questions they should ask before buying any supplement online in India.
4. What food sources in an Indian diet already cover the same ground.
5. What to say when the client has already bought it.

No brand names. No doses. No claims I cannot verify - write 'not established' where that is the truth.

Note *'Not established' is a complete answer and a professional one. Never let a document imply a supplement does something the evidence does not support.*

14 The 'no results' honest conversation

USE WHEN *Week six, and the client is right to be unhappy.*

My client is not seeing the results they expected. They are [number] weeks in.

WHAT THEY REPORT: [their account]
WHAT I SEE: [what the records show]
WHAT WAS PROMISED: [what I actually said at the start, as accurately as I can]

Give me:
1. An opening that does not blame them and does not over-apologise.
2. How to separate three possibilities honestly: the plan, the follow-through, or the expectation.
3. What to say if it is my plan that was wrong.
4. What to say if the expectation was unrealistic - without sounding like an excuse.
5. A revised plan for the next four weeks, in three bullets.
6. What to offer if they want to stop.

Under 300 words, spoken voice, no defensiveness.

Note *Owning a wrong plan in one clean sentence buys more trust than a perfect record. Clients forgive error, not evasion.*

PART 5 OF 10

Client Handouts and Education

The writing you repeat every week: explaining the same condition, the same label, the same myth to a different person. These prompts give you a first draft in a minute, in language an Indian client will actually read and forward to their family.

14 PROMPTS IN THIS PART

01 The one-page handout builder

02 The insulin resistance explainer

03 The iron and anaemia handout for vegetarians

04 Vitamin D and B12

05 The gut health and bloating handout

06 The cholesterol explainer

07 The pregnancy nutrition handout

08 Post-partum and breastfeeding

09 The senior nutrition handout (60+)

10 The board-exam-year teen plan

11 Eating out at an Indian restaurant

12 Reading an Indian food label

13 The myth-buster set

14 The blood pressure and salt handout

1 The one-page handout builder

USE WHEN *The master prompt. Use it for any topic.*

```
Create a one-page client handout on [topic] for my nutrition practice in India.

AUDIENCE: [who they are, education level, language comfort]
WHAT THEY ALREADY BELIEVE ABOUT IT: [the misconception to address]
FORMAT: single page, printable, and readable on a phone

Structure:
- Two-sentence opening in plain language.
- 'What this means for you' - three lines, no jargon.
- 'Try this' - five specific steps, each starting with a verb, each using Indian food.
- 'Go easy on' - three lines, no food described as bad or forbidden.
- 'When to see your doctor' - two lines.
- One closing line that is encouraging and true.

Under 350 words. No statistics unless I give them to you. No calorie figures. Grade 7 reading level.
```

Note *One prompt, thirty handouts. Keep the 'no food is bad' rule - it is what stops the handout from creating disordered eating in a vulnerable client.*

2 The insulin resistance explainer

USE WHEN *The concept behind half your client list.*

```
Explain insulin resistance to my client in simple Indian English.

CLIENT: [age, what they know already, why they were told to see me]

Give me:
1. A 150-word explanation using one everyday comparison - a door, a key, a delivery, anything visual. Not a medical diagram.
2. What it feels like in daily life - the 3 pm slump, the cravings, the tiredness after a heavy lunch.
3. The three food changes that help most, in Indian food terms.
4. What does NOT help, even though people believe it does.
5. One line on why this is not their fault.
6. One line on what is in their hands.

Under 300 words. No lab values, no percentages.
```

Note *The 'not their fault' line is not softness - it is what makes the client able to act. Shame is a poor motivator for a 12-week behaviour change.*

3 The iron and anaemia handout for vegetarians

USE WHEN *Enormously common, badly understood.*

Write a handout on iron for a vegetarian client in India.

CLIENT: [age, sex, whether they have been told they are anaemic, what they eat, region]

Cover:
1. Why iron is harder on a vegetarian diet, in two honest sentences.
2. The best vegetarian iron sources in an Indian kitchen, with the everyday portion described in katoris or spoons - not grams.
3. The pairing rule that increases absorption, and the two things that block it - with specific Indian examples (tea, coffee, calcium, phytates).
4. A sample day that puts the pairing to work.
5. What NOT to do - including self-medicating with supplements.
6. When they must see a doctor rather than change their food.

Under 400 words. No supplement doses. No claims that food alone fixes diagnosed anaemia.

Note *The pairing rule (iron plus vitamin C, away from tea) is the single most useful thing you can teach an Indian vegetarian client.*

4 Vitamin D and B12

USE WHEN *The two deficiencies almost every Indian client has been told about.*

Write a handout on vitamin D and vitamin B12 for my Indian clients.

CONTEXT: [typical client profile - city, indoor work, vegetarian, sunscreen use, sun exposure]

Cover, for each vitamin separately:
1. Why it is commonly low in India, in two sentences - and why 'we have so much sun' does not protect people.
2. The food sources that actually matter in an Indian diet, and an honest note on how much food alone can do.
3. What the client should ask their doctor - the test, the follow-up test, and the timing.
4. What I, as a dietitian, will not do (prescribe a dose).
5. Three practical food habits to add.

Under 400 words. No doses, no brand names, no claims about curing fatigue.

Note *Never put a dose in a handout. The moment a client reads 'take 60,000 IU weekly' in your document, that is your advice, not their doctor's.*

5 The gut health and bloating handout

USE WHEN *The most-Googled complaint in Indian nutrition.*

Write a handout on bloating and gut comfort for an Indian client.

CLIENT: [age, what they describe, whether it is daily or after specific meals, diet type, region]

Cover:
1. What bloating usually is and is not, in three honest lines.
2. The five most common Indian food triggers, described as 'worth testing' rather than 'causes'.
3. How to run a simple two-week test - remove one thing at a time, and what to record.
4. The habits that help regardless of trigger: eating speed, meal spacing, water, walking.
5. What to add - the Indian foods that support the gut and are cheap.
6. The red flags that mean a doctor, not a diet change: blood, unexplained weight loss, night pain, fever.

Under 400 words. No probiotic brand names. No 'leaky gut' claims.

Note *The red-flag list is the reason this handout is safe to hand out. Blood, night pain and unexplained weight loss are never a diet problem.*

6 The cholesterol explainer

USE WHEN *Clients who have been told 'cut ghee and oil' and nothing else.*

Explain cholesterol and lipids to my Indian client.

CLIENT: [age, what their report shows as they understand it, diet type, whether they cook with ghee, coconut oil, mustard oil or refined oil]

Give me:
1. A plain explanation of the numbers on their report, in under 180 words - which ones matter and which are context.
2. What actually moves each number, and how much of it is food versus other things.
3. The Indian cooking-fat conversation, honestly - ghee, coconut oil, mustard oil, refined oil - with the one swap that helps most.
4. The fibre angle: which Indian foods do the most work here.
5. What food cannot fix, and must stay with their doctor.
6. Three questions to ask at their next appointment.

No claims that diet replaces statins. No numbers I have not given you.

Note *Being honest that food cannot replace a statin is what makes the diet advice trustworthy. Clients can smell an agenda.*

7 The pregnancy nutrition handout

USE WHEN *Only for clients whose antenatal care is with a doctor.*

Write a nutrition handout for a client in [trimester] of pregnancy, whose antenatal care is with an obstetrician.

CLIENT: [age, diet type, region, food aversions, any condition, whether they work]

Cover:
1. What changes in each trimester, food-wise, in three lines.
2. The Indian foods to prioritise - iron, protein, calcium, folate - with everyday portions.
3. Foods to avoid, and why, in one line each - including the raw and unpasteurised items that matter in India.
4. How to handle nausea, aversions and cravings.
5. What the client must ask their doctor rather than me: supplements, weight gain targets, any complication.
6. One line about family pressure to 'eat for two'.

Under 450 words. State clearly at the top that this is supportive nutrition guidance alongside medical care, not a substitute.

Note *The header line is not decoration. Pregnancy is the one context where an unsourced document can do real harm.*

8 Post-partum and breastfeeding

USE WHEN *A client group that gets almost no structured support in India.*

Write a nutrition handout for a client who is [number] weeks post-partum and [breastfeeding / not breastfeeding].

CLIENT: [delivery type if they share it, sleep situation, who helps at home, diet type, region, goal]

Cover:
1. What the body needs now, in three plain lines - with the honest note that this is not the time for aggressive weight loss.
2. The Indian post-partum foods that genuinely help - and a respectful note on traditional foods, separating what is useful from what is only custom.
3. How to eat when there is no time to cook: five things that take under five minutes.
4. Hydration and the one habit most mothers skip.
5. What to do about the pressure to 'get back to shape' from family or social media.
6. What to refer to their doctor.

Under 400 words. Warm, non-judgemental, and honest about the sleep problem.

Note *Separating 'useful tradition' from 'custom only' - respectfully - is a skill almost no Indian handout has. It is why clients trust this one.*

9 The senior nutrition handout (60+)

USE WHEN *Parents and grandparents, usually brought in by their children.*

```
Write a nutrition handout for a client over 60.

CLIENT: [age, living situation, diet type, region, medicines they take, dental or swallowing issues, who cooks]

Cover:
1. What changes with age and matters most: protein, bone health, muscle, hydration, appetite.
2. The protein problem in Indian senior diets, and five easy additions to food they already eat.
3. Texture and appetite: what to do when chewing is difficult or food tastes dull.
4. The hydration reminder that actually works in an Indian household.
5. What to do about the medicines-food timing question - refer, do not answer.
6. A simple daily checklist in five ticks.

Under 400 words. Large-type friendly, short sentences, no jargon. Written so an adult child can read it aloud.
```

Note *Write this one for the son or daughter, not the parent. They are the ones who will print it and stick it on the fridge.*

10 The board-exam-year teen plan

USE WHEN *Class 10 and 12 students, usually brought by a worried parent.*

```
Write a nutrition handout for a student in class [10 / 12] during the exam year.

CONTEXT: [board, study hours, coaching schedule, hostel or home, diet type, sleep pattern, caffeine use]

Cover:
1. What food does for attention and memory - honestly, without promising marks.
2. A study-day eating structure: what to eat before a study block, at the 3 pm dip, and before sleep.
3. The five things students in India actually consume during exams, and what to do about each.
4. Caffeine, energy drinks and the crash - one clear paragraph.
5. Sleep and screens, in three lines.
6. What parents should stop saying, written for the parent to read.

Under 400 words. Speak to the student, not over them. No supplement advice.
```

Note *The last section - what parents should stop saying - is why this handout gets shared in school WhatsApp groups. Build for the sharer.*

11 Eating out at an Indian restaurant

USE WHEN *Attach to every plan. Clients use it more than the plan itself.*

Write a practical guide to eating out at Indian restaurants for my client.

CLIENT: [goal, diet type, region, how often they eat out, whether they drink alcohol]

Cover:
1. How to order at a North Indian restaurant - the order to place the dishes in, what to skip, what to ask for.
2. The same for a South Indian restaurant, a biryani or Mughlai place, a street-food outing, and a cafe.
3. The three requests a client can make to the kitchen that are almost always honoured.
4. What to do about the bread, the rice, the fried starters and the sweet - in that order.
5. How to handle a family meal where they are not choosing the dishes.
6. One line on alcohol, if they drink.

Under 400 words, practical, no food called 'bad', no calorie figures.

Note *The 'three requests the kitchen honours' idea is what makes this usable. Clients remember requests; they forget rules.*

12 Reading an Indian food label

USE WHEN *For clients buying packaged food - which is nearly all of them.*

Teach my client to read an Indian packaged-food label.

CLIENT: [goal, literacy level, what they usually buy, whether they read Hindi or English]

Cover:
1. Where to look first on the pack, and why the front of the pack is marketing.
2. The four numbers on the nutrition panel that matter for their goal, and what counts as high or low - with a note that they should compare similar products rather than memorise cut-offs.
3. How to read the ingredients list in order, and the three names that hide sugar.
4. Serving size versus the whole pack - the most common trap in India.
5. The claims to distrust: 'no added sugar', 'made with real fruit', 'multigrain', 'lite'.
6. A 30-second routine they can do in a shop aisle.

Under 400 words. No brand names. No numbers you cannot verify.

Note *'Multigrain' and 'no added sugar' are the two claims that fool Indian buyers most. Naming them is the value of this page.*

13 The myth-buster set

USE WHEN *Five corrections you give every month anyway.*

```
Write five myth-correction cards for my Indian clients.

Each card covers one common belief from this list, or one you consider equally common in India:
ghee makes you fat; rice at night causes weight gain; jaggery and honey are healthy sugars; skipping breakfast is fine or fatal; lemon water in the morning burns fat.

For each card:
1. The belief, stated fairly - as the client actually says it.
2. What is actually true, in two sentences, with the part that is right acknowledged first.
3. The practical takeaway for an Indian kitchen.
4. A one-line version for a WhatsApp status.

Under 60 words per card. No mocking, no 'studies show' without a real source, no absolute bans.
```

Note *Acknowledging the true part first is what stops the client dismissing the correction as 'dietitian talk'.*

14 The blood pressure and salt handout

USE WHEN *Extremely common, and Indian diets hide salt in places clients never check.*

```
Write a handout on salt and blood pressure for an Indian client.

CLIENT: [age, whether they are on medication, who cooks, region, how much packaged food they eat, whether they add salt at the table]

Cover:
1. What salt does to blood pressure, in three plain lines - no scare language.
2. The five places salt hides in an Indian kitchen and diet: pickle, papad, packaged masala, biscuits and namkeen, and the salt added at the table.
3. A realistic reduction plan over four weeks, one change at a time, that does not make food taste like nothing.
4. The swaps that keep flavour - amchur, lemon, kokum, tamarind, roasted jeera, black pepper, curry leaf, hing.
5. What to do about eating out and about the food cooked by someone else.
6. Why they must not stop or change any blood pressure medicine on their own.

Under 400 words. No numbers I have not given you. No claim that diet replaces medication.
```

Note *The medicine warning is not optional here. Salt advice without it is the one handout that could genuinely hurt someone.*

PART 6 OF 10

The Content Engine

Twenty-five thousand nutritionist accounts exist on Indian Instagram and most of them have weaker credentials than you. Content is how the qualified practitioner stops losing clients to the louder one. This part builds a month of content in an hour.

12 PROMPTS IN THIS PART

01 Your content pillars

02 The 30-day calendar

03 The 30-second reel script

04 One idea, five slides

05 The myth-buster post, done responsibly

06 The client-win post, with consent

07 The hook bank

08 The local SEO page outline

09 Google Business profile and review replies

10 The WhatsApp broadcast that starts conversations

11 The workshop or webinar outline

12 The newsletter issue

1 Your content pillars

USE WHEN *Once. Then everything else follows from it.*

Build my content pillars as a dietitian in India.

MY NICHE: [e.g. PCOS and insulin resistance for women 25-40 in Bengaluru]
MY CLIENTS: [who they are, what they worry about, what they have tried]
WHAT I AM CREDENTIALED IN: [qualifications, registration, specialties]

Give me:
1. Four content pillars that fit my niche, each with a one-line purpose and the type of client it attracts.
2. For each pillar, five post topics that a qualified professional can speak on but an influencer cannot.
3. The pillar I should post most often, and why.
4. Three topics I should avoid entirely, given my scope of practice.
5. A one-line bio for my Instagram profile that leads with credentials and ends with who I help.

No hype, no 'transform your life' language, no medical claims.

Note *Credential-led positioning is the only durable edge over the 25,000 unqualified accounts. Build the pillars on what you can defend.*

2 The 30-day calendar

USE WHEN *One hour, once a month.*

Build a 30-day content calendar for my nutrition practice.

PILLARS: [the four from the previous prompt]
PLATFORM: [Instagram / YouTube Shorts / both]
CAPACITY: I can produce [number] posts a week and record [number] videos a month
WHAT IS COMING UP: [festivals, seasons, exam months, wedding season, anything relevant]

Give me a table: date, format (reel / carousel / single / story / text post), pillar, working title, the one point the post makes, and the call to action.
Rules:
- Mix education, myth-correction, behind-the-practice, and one offer post a fortnight - no more.
- Include two posts that are repurposed from the same idea.
- Include one post that answers a question clients actually asked me this month.
- Keep every post within my scope of practice.

Then tell me which three posts to record first, and why.

Note *One offer post a fortnight is the discipline that keeps a health account from becoming a shop. Indian audiences unfollow quickly when every post sells.*

3 The 30-second reel script

USE WHEN *For any single idea.*

Write a 30-second reel script for my nutrition practice.

TOPIC: [the idea]
AUDIENCE: [who it is for]
MY ANGLE: [the qualified position, in one line]

Format:
- 0-3 seconds: a hook that a scrolling Indian viewer will not swipe past - specific, not clickbait.
- 3-8 seconds: the problem, in the viewer's words.
- 8-22 seconds: the one point, with one Indian food example.
- 22-27 seconds: the practical takeaway.
- 27-30 seconds: a soft close, no hard sell.

Give it as: shot, what I say, and what appears on screen. Mark where the text changes. Under 110 spoken words. No claims I cannot defend, no invented statistics.

Note *Ask for the on-screen text separately - most Indian viewers watch muted, so the text is doing half the work.*

4 One idea, five slides

USE WHEN *Carousels outperform almost everything for a professional account.*

Turn one idea into a five-slide Instagram carousel.

IDEA: [the idea]
AUDIENCE: [who]

Slide 1: a hook that names the problem in the client's own words - under 12 words.
Slides 2-4: one point each, with one Indian food example per slide, under 30 words per slide.
Slide 5: the takeaway plus a soft call to action, under 25 words.

Also give me: the caption (under 120 words, first line works as a standalone hook), five hashtags that are specific rather than huge, and the alt text for accessibility.

No medical claims, no numbers I cannot verify, no before-after framing.

Note *Alt text is a small professionalism signal that almost no Indian nutrition account bothers with. It also helps reach.*

5 The myth-buster post, done responsibly

USE WHEN *High reach, high risk. This prompt manages both.*

```
Write a myth-busting post for my nutrition practice.

THE MYTH: [what people believe]
WHY IT SPREADS: [why it feels true]
WHAT IS ACTUALLY TRUE: [my position]

Give me:
1. A hook that states the myth fairly, without ridiculing the people who believe it.
2. The true part of the myth, acknowledged in one line.
3. The correction, in three short points, with one Indian food example each.
4. What the viewer should do differently this week - one action.
5. A closing line that is confident without being absolute.
6. A caption version and a 30-word story version.

Explicitly avoid: 'studies show', absolute language like 'never' or 'always', and any claim about curing a condition.
```

Note *'Studies show' with no study is the fastest way to lose professional credibility. Ban the phrase from your account permanently.*

6 The client-win post, with consent

USE WHEN *The highest-converting format, if it is done properly.*

```
Write a post about a client's progress.

THE STORY: [what happened, what changed, over how long]
CONSENT: [what the client agreed to - name or initials, city, photo, numbers]
WHAT I WILL NOT SHARE: [the details that stay private]

Give me:
1. A post that tells the story without promising the same result to the reader.
2. Where to place the line that says results vary and depend on the individual.
3. How to describe the change without reducing it to a number on a scale.
4. A version with no numbers at all, if consent does not cover them.
5. The caption and a short story version.

No before-after body framing, no shame language, no 'she transformed'.
```

Note *Results-vary is not legal boilerplate - it is the difference between honest marketing and a complaint. Put it in the post, not the comments.*

7 The hook bank

USE WHEN *Twenty openers you can reuse for months.*

Write 20 opening lines for nutrition content aimed at [audience] in India.

Mix these angles: the specific mistake, the thing nobody tells you, the local reality (Indian food, family, festivals, office), the honest admission, the question clients actually ask, and the myth stated fairly.

Rules:
- Under 12 words each.
- No clickbait that the post cannot deliver on.
- No 'doctors hate this', no fear, no body shaming.
- At least five must mention a specific Indian food or situation.
- At least three must be usable by a dietitian specifically, not a general wellness account.

Then mark the five you think will perform best and say why in one line each.

Note *Hooks that promise something the post cannot deliver cost you followers who arrive and leave. Ask for the deliverable check.*

8 The local SEO page outline

USE WHEN *The highest-intent traffic in Indian dietetics: '[dietitian in [city]'.*

Outline a webpage for my practice targeting the search '[dietitian / nutritionist] in [city]'.

MY PRACTICE: [location, mode - online, clinic or both, specialties, languages, credentials]

Give me:
1. A title tag and meta description within length limits.
2. The H1 and the section headings in order, with one line on what each section must prove.
3. The five questions a person searching this phrase actually wants answered.
4. What local detail to include that a generic page cannot - areas served, languages, clinic timing, how to reach.
5. The trust elements to place above the fold.
6. What to leave out, because it reads as padding to a search engine and a human.

No keyword stuffing. No invented reviews, awards or patient numbers.

Note *Never let AI write reviews or patient counts. Point 6 is the one that keeps the page honest and readable.*

9 Google Business profile and review replies

USE WHEN *Where most local clients actually find you.*

Write the text for my Google Business profile as a dietitian in [city].

MY PRACTICE: [services, mode, languages, timings, specialties]

Give me:
1. A business description under 750 characters that leads with who I help and where.
2. Five service entries with a one-line description each.
3. Three posts for the profile - one introducing the practice, one about a common condition, one about a festival or season.
4. Reply templates for: a five-star review, a three-star review with a specific complaint, and a review that contains a medical claim I cannot endorse.
5. What I must never say in a review reply.

No medical claims in any of it. No promises of results.

Note *The three-star reply is the one that wins new clients - everyone reads the negative reviews first.*

10 The WhatsApp broadcast that starts conversations

USE WHEN *Monthly, to your existing list. Never daily.*

Write a WhatsApp broadcast to my client and enquiry list.

WHAT I WANT TO SAY: [the topic or offer]
MY LIST: [size, how they know me, mostly what kind of client]
SEASON: [what is happening this month in India]

Give me:
1. A broadcast under 120 words that reads like a message from a person, not a brand.
2. A first line that earns the next line.
3. One useful thing in the message, even for someone who never buys.
4. One clear action, with a single link or reply instruction.
5. A version for clients already on a programme, which does not sell to them.
6. The opt-out line, written plainly.

No emoji spam, no 'Dear client', no false urgency, no discount invented by you.

Note *The non-selling version for existing clients is the one that keeps the list alive. Broadcast lists die when every message is a pitch.*

11 The workshop or webinar outline

USE WHEN *A free session that fills a paid programme.*

Build a 45-minute workshop outline on [topic] for [audience] in India.

Give me:
1. A title that names the outcome, not the topic.
2. The opening three minutes, including the promise and the one thing they will leave with.
3. Three teaching blocks, each with one activity and one Indian food example.
4. The Q&A; plan - the five questions that always come up, and the short answers.
5. The transition to my programme, done without pressure.
6. A one-page handout to send afterwards.
7. What to do in the last two minutes to convert.

No medical claims. No promises of results. Keep it deliverable by one person on a video call.

Note *The 'what they leave with' promise in the first three minutes is what stops people dropping off at minute ten.*

12 The newsletter issue

USE WHEN *Monthly. One issue, one idea.*

Write a monthly newsletter issue for my nutrition practice.

THIS MONTH'S IDEA: [one idea]
AUDIENCE: [clients and subscribers in India]
WHAT IS HAPPENING: [season, festival, exam period]

Give me:
1. A subject line under 50 characters and a preview line under 90.
2. An opening of three sentences that names something they are experiencing right now.
3. One idea explained properly - under 300 words, with one Indian food example.
4. One practical thing to try this week.
5. One short section answering a question a client asked me.
6. A closing with one link and no hard sell.

No filler, no 'hope you are doing well', no listicle padding. If the issue is thin, tell me it is thin.

Note *'Tell me if it is thin' is a useful instruction. It stops the model padding a single idea into a hollow 800-word issue.*

PART 7 OF 10

Practice, Pricing and Money

The business half that no nutrition degree teaches. Positioning, packages, the discovery call, the corporate pitch and the numbers you need to know to price properly in the Indian market.

12 PROMPTS IN THIS PART

01 The positioning statement
02 The four-tier package sheet
03 The discovery call script
04 The package page copy
05 The profile and link-in-bio
06 The corporate wellness pitch
07 The doctor referral letter
08 The gym and trainer partnership
09 The price increase message
10 The monthly revenue review
11 The simple P&L;
12 The GST and invoicing note

1 The positioning statement

USE WHEN *Before you touch pricing or marketing.*

Build my positioning statement as a dietitian in India.

MY CREDENTIALS: [qualifications, registration, years of experience, specialties]
MY CLIENTS SO FAR: [who they have been, what they came for]
WHAT I ENJOY: [the work you want more of]
MARKET: [city or online, competition you see]

Give me:
1. Three positioning options, each in one sentence: 'I help [who] with [problem] using [approach]'.
2. For each, who it attracts and who it repels.
3. Which one is most defensible given my credentials, and why.
4. The proof I need to build for the option you recommend - what to collect over the next 90 days.
5. The one sentence I should remove from my current messaging.

No invented achievements. Use only what I have given you.

Note *Positioning that repels someone is working. If it attracts everyone, it says nothing.*

2 The four-tier package sheet

USE WHEN *The core commercial document of your practice.*

Design my programme packages.

MY TIME: [hours available per week for client work]
MY CURRENT FEES: [what I charge now, per session and per month]
MY COSTS: [room rent, software, assistant, marketing spend]
WHAT CLIENTS ASK FOR MOST: [the requests]

Design four tiers, and for each give: name, who it is for, duration, what is included (sessions, WhatsApp support, reviews, documents), the monthly price, and what is deliberately NOT included.
Rules:
- The entry tier must be genuinely useful on its own, not a teaser.
- The top tier must be priced for the effort it actually takes.
- Show the price as both monthly and per-day.
- Then tell me my effective hourly rate at each tier if I sell [number] of each per month, and which tier I should push.

Prices in rupees. Do not invent market data - use my numbers.

Note *Working out the effective hourly rate per tier is the step almost every Indian dietitian skips, and it is where the unprofitable package hides.*

3 The discovery call script

USE WHEN *The 15-minute call before anyone pays.*

```
Write my 15-minute discovery call script as a dietitian in India.

MY OFFER: [the packages]
TYPICAL ENQUIRY: [who calls, what they ask first]

Structure it as: what I say, what I ask, what I listen for, and what I write down.
Cover:
1. The 60-second opening that sets the frame - this is a conversation, not a consultation.
2. The five questions that tell me whether this client is right for my programme.
3. How to say 'I am not the right person for this' when it is true.
4. How to present the package without reading a list.
5. The close - what I say to move to payment, plainly.
6. What to send within an hour of the call.

No pressure tactics. No fake scarcity.
```

Note *Saying 'I am not the right person' out loud is the fastest way to build a reputation that generates referrals.*

4 The package page copy

USE WHEN *The page or PDF you send after the call.*

```
Write the copy for a one-page description of my programme.

PROGRAMME: [name, duration, what is included, price]
CLIENT: [who it is for]
OUTCOME: [what they will have at the end - described as capability, not a number]

Write:
1. A headline in outcome language.
2. Three lines on who this is for and who it is not for.
3. What happens in week 1, week 2-4, and the rest - in the client's terms.
4. What is included, as a plain list.
5. What is not included, stated clearly.
6. The price, and the per-day equivalent.
7. The two questions to answer before they pay.
8. The next step, in one instruction.

No guarantees of weight loss. No 'limited seats' unless it is literally true.
```

Note *Saying who it is not for is what makes the page believable. A page for everyone converts nobody.*

5 The profile and link-in-bio

USE WHEN *Your shopfront.*

Write my Instagram and Google profile copy as a dietitian in India.

CREDENTIALS: [what I can legitimately claim]
NICHE: [who I help with what]
MODE: [online / clinic / both], LANGUAGES: [languages]
OFFER: [the entry package]

Give me:
1. Three bio options under 150 characters, each leading with credibility and ending with who I help.
2. A link-in-bio page structure with five links in priority order and a one-line label each.
3. A pinned-post idea that answers 'who are you and why should I trust you'.
4. The first three story highlights to create, and what goes in each.
5. What to remove from my profile because it undermines credibility.

No claims beyond my credentials. No 'DM for collabs' filler.

Note *Leading with credentials is a deliberate choice for the Indian market, where the buyer cannot tell a registered dietitian from an Instagram coach.*

6 The corporate wellness pitch

USE WHEN *The highest-value, slowest-cycle revenue in Indian dietetics.*

Write a corporate wellness pitch from me, a dietitian in India, to an HR head.

TARGET COMPANY: [size, sector, city, anything I know about them]
WHAT I OFFER: [the programme - sessions, workshops, screenings, duration, price band]
WHY THEM: [the specific reason I am approaching this company]

Give me:
1. A subject line and a 120-word email that gets a reply, not a brochure.
2. The one-line value proposition framed for HR - attendance, energy, claims, retention - not for the employee.
3. A one-page proposal structure with headings.
4. Three objections HR will raise, and my answer to each.
5. The follow-up sequence - two emails, spaced, not five.
6. What I must not claim without data.

No invented statistics about productivity or absenteeism. No 'wellness revolution' language.

Note *Never quote a productivity statistic you cannot source. HR teams check, and one invented number ends the conversation.*

7 The doctor referral letter

USE WHEN *The most reliable client pipeline in Indian dietetics.*

```
Write an introduction letter from me, a dietitian, to a doctor I would like to work with.

DOCTOR: [specialty, hospital or clinic, city]
WHY THEM: [the patients they see that I could help]
WHAT I DO: [my credentials, specialties, how I work, mode]

Give me:
1. A one-page letter, formal but human, under 250 words.
2. The specific patient type I can take off their hands, described clinically.
3. How I will communicate back - the note format, the frequency.
4. What I am NOT asking for - stated plainly, so it does not read as a request for a cut.
5. A one-paragraph version for WhatsApp, if the letter is too much.
6. The three things I should bring to the first meeting.

No claims about outcomes. No mention of commissions or referral fees - ever.
```

Note *Stating what you are not asking for is what makes a doctor read the second letter. Never, ever offer a cut - it is unethical and it ends the relationship.*

8 The gym and trainer partnership

USE WHEN *Where a lot of Indian diet clients actually come from.*

```
Write a partnership proposal from me, a dietitian, to a gym owner or personal trainer in India.

THE GYM: [location, size, client type, what they offer]
WHAT I PROPOSE: [the arrangement - referral both ways, in-house slots, a combined package]

Give me:
1. A short message that opens with their problem, not my service.
2. Three partnership models, from lightest to deepest, with what each needs from them.
3. How to handle the money conversation without an awkward revenue split discussion.
4. The one thing that makes trainers trust a dietitian - and the one thing that makes them stop referring.
5. A follow-up message for silence.
6. What I should refuse, even if it costs me the partnership.

Under 250 words for the opening message. No commission language unless I raise it.
```

Note *Trainers stop referring the moment a dietitian overrides them in front of a client. Point 4 is worth the whole prompt.*

9 The price increase message

USE WHEN *Once a year, and to existing clients.*

I am raising my fees from Rs [old] to Rs [new], effective [date]. Write what I send to existing clients.

MY CLIENTS: [how long they have been with me, how many, their price sensitivity]

Give me:
1. The message, under 150 words, that states the new price once and clearly.
2. What I am adding, if anything, in exchange.
3. What happens to clients currently mid-programme - the honest, fair position.
4. What to say to the client who says they cannot afford it.
5. What not to say - the three justifications that make it sound like an apology.
6. The version for a client I want to keep at the old price.

No invented discounts. No fake 'costs have gone up' claims unless they have.

Note *Saying the number once, without justification, is the whole skill. Every extra sentence sounds like an apology.*

10 The monthly revenue review

USE WHEN *The last Sunday of every month.*

Run a monthly review of my nutrition practice.

MY NUMBERS: [enquiries, discovery calls, clients signed, clients lost, average fee, total revenue, hours worked, marketing spend]
MY GOALS: [the number I want to hit]

Give me:
1. The three numbers that actually matter this month, and why.
2. Where the leak is: enquiries to calls, calls to clients, or clients to renewals.
3. One change to make next month, not five.
4. My effective hourly rate, and what it says.
5. What I should stop doing entirely.
6. One question I am avoiding.

Plain language. If the numbers are too thin to conclude anything, say so instead of guessing.

Note *'One change, not five' is the discipline. Practices die from trying to fix everything in the same month.*

11 The simple P&L;

USE WHEN *Quarterly. You cannot price without it.*

Build a simple profit and loss for a solo nutrition practice in India.

MY INPUTS: [monthly revenue, room rent, software subscriptions, assistant cost, marketing spend, travel, professional fees, taxes paid, equipment bought this quarter]

Give me:
1. A clean monthly P&L; with the categories I gave you and the ones I probably forgot.
2. My gross margin and my real take-home, with the difference explained.
3. The three costs that are growing faster than revenue.
4. What to cut and what to keep, with one line of reasoning each.
5. The single number I should track weekly.
6. What to hand to my CA at the end of the quarter.

No tax advice - flag anything that needs my chartered accountant and stop there.

Note *Flagging-and-stopping is the correct behaviour on tax. A dietitian's AI document must never read like tax advice.*

12 The GST and invoicing note

USE WHEN *When clients start asking for invoices.*

Explain, for my own understanding as a dietitian in India, how invoicing and GST might apply to my practice.

MY SITUATION: [solo or with staff, revenue band, mode - online or clinic, whether I am registered for GST, client types]

Give me:
1. The questions I must ask a chartered accountant, phrased precisely, so the conversation takes ten minutes instead of an hour.
2. What a clean invoice should contain regardless of tax position.
3. What clients usually need on an invoice for reimbursement or company claims.
4. The records to keep, and for how long, without guessing at legal specifics.
5. Where I should not rely on an AI answer at all.

Do not state GST rates, thresholds or legal requirements as fact. Frame every point as a question for my CA. This is not tax advice.

Note *This prompt is deliberately built to produce questions, not answers. Tax rules change and a wrong rate in your invoice is a real problem.*

PART 8 OF 10

Records, Data and Scope

The part that keeps your practice safe. Client records, consent, anonymisation, the note to a doctor and the protocol for when AI gets something wrong. Indian client health data is personal data, and this is the discipline that respects it.

10 PROMPTS IN THIS PART

01 The SOAP note from a session
02 The progress note template
03 The consent and scope note
04 The data-minimisation checklist
05 The handover summary to a doctor
06 The incident log
07 The discharge summary
08 The records retention plan
09 The AI error correction protocol
10 The scope-of-practice script

1 The SOAP note from a session

USE WHEN *Within an hour of every consultation.*

Turn my session notes into a structured clinical record.

MY RAW NOTES: [paste your scribbles]
SESSION: [first consultation / follow-up number X], date [date]

Format as a professional nutrition record:
S - Subjective: what the client reported, in their words where possible.
O - Objective: measurements I took, records they sent. No numbers I did not give you.
A - Assessment: my working assessment, in my own framing - do not add a diagnosis.
P - Plan: what we agreed, the next check-in, what they will send before it.

Rules:
- No invented measurements, no invented history.
- No diagnostic language that is not already in my notes.
- Flag anything that should go to their doctor as a separate line.
- Keep it under 350 words.

Then give me a two-line summary suitable for a handover.

Note *Ask it to flag rather than diagnose. A note that says 'consider medical review' is defensible; a note that names a disease is not.*

2 The progress note template

USE WHEN *Build once, use for every follow-up.*

Create a progress note template for my nutrition follow-ups.

WHAT I TRACK: [measurements, symptoms, adherence, food records, wellbeing]
HOW OFTEN: [weekly / fortnightly / monthly]
WHAT I NEED IT FOR: [my own record, handover to a doctor, or both]

Give me:
1. A one-page template with labelled fields, in the order I would actually fill them.
2. The three fields that must never be left blank, and why.
3. A short free-text section for what changed and what did not.
4. A section for what I decided NOT to change this session, and why - because that decision matters later.
5. A tick-box line for 'referred to doctor: yes/no/not needed'.
6. How to keep the whole thing fillable in under four minutes.

Note *Recording what you deliberately did not change is what makes a record useful six months later - and it is what most notes omit.*

3 The consent and scope note

USE WHEN *Once, at onboarding, in writing.*

Draft the consent and scope-of-practice note I give every new nutrition client in India.

MY PRACTICE: [credentials, mode, what I do and do not do, how I store records, how they can reach me]

Write it in plain English, under 350 words, covering:
1. What my service is and what it is not - in the client's language, not legal language.
2. That I do not diagnose, prescribe, or change medication, and that they must continue with their doctor.
3. What information I collect and why, kept to the minimum I actually need.
4. Who sees it, where it is stored, and for how long.
5. How they can ask for a copy or ask me to delete it.
6. That they can stop at any time.
7. What happens if I need to share information with their doctor.
8. How they give consent - one clear line.

No legalese. Flag anything that should be reviewed by a lawyer before I use it.

Note *This is a working draft for a lawyer to review, not a finished legal document - and the prompt says so. That framing matters.*

4 The data-minimisation checklist

USE WHEN *Before you paste anything into any AI tool.*

Give me a checklist for preparing a client's information before I use an AI assistant.

MY TOOLS: [which AI assistants I use, whether work or personal accounts, whether the tool trains on inputs]

Produce:
1. The strip list: the categories of detail that must be removed every time (names, contact details, exact dates of birth, addresses, employer names, record numbers, photos, lab report identifiers).
2. The keep list: what the AI actually needs to be useful - and how to phrase it so it stays useful.
3. A before-and-after example: a realistic Indian client profile, then the same profile anonymised, side by side.
4. The three mistakes people make when anonymising (keeping the city plus the profession plus the age, which re-identifies).
5. What to do if I have already pasted something I should not have.
6. A 30-second routine to run before every paste.

Do not give legal advice about Indian data law. Give me operational discipline.

Note *The re-identification point is the one people miss: '34-year-old paediatrician in Kohima with PCOS' is a name.*

5 The handover summary to a doctor

USE WHEN *When a client's care needs to be joined up.*

Write a handover summary from me, a dietitian, to the client's treating doctor.

CLIENT: [age band, condition, what the client reported, what I have done nutritionally, how long]
WHY I AM WRITING: [a specific clinical question, or 'to keep you informed']

Give me:
1. A one-page summary in the order a doctor reads: reason for contact, relevant history as reported, what I have implemented, what has changed, my question, my contact.
2. The parts to keep to one line each so it fits a busy clinic.
3. What to leave out entirely - my opinions, the client's emotional context, marketing language.
4. A version under 150 words for a WhatsApp message to the doctor's clinic.
5. The line that makes clear I am not asking them to endorse my plan.

No diagnosis, no medication comment, no claims about outcomes.

Note *A doctor reads the first two lines and decides whether to keep reading. Reason for contact first, always.*

6 The incident log

USE WHEN *The day something goes wrong. Write it the same day.*

Help me write an incident record for my nutrition practice.

WHAT HAPPENED: [what occurred, when, who was involved, what the client reported]
WHAT I DID: [your actions, in order, with times]
WHAT IS OPEN: [what still needs to happen]

Give me:
1. A factual, neutral, chronological record - no self-justification, no blame.
2. The separation of fact from interpretation, clearly labelled.
3. What to record about communication with the client and with any doctor.
4. What to do in the next 24 hours, as a short list.
5. What NOT to write down - speculation, opinion about the client's character, anything I would not want read out.
6. When to escalate to a senior professional or a lawyer.

Under 400 words. Plain, calm, dated.

Note *Write it the same day, in facts. A record written a week later, in defensiveness, is worse than no record.*

7 The discharge summary

USE WHEN *When a programme ends well, or ends early.*

```
Write a discharge summary for a client finishing with me.

PROGRAMME: [duration, what we worked on]
WHAT CHANGED: [the changes, in the client's terms]
WHAT THEY WILL KEEP DOING: [the habits that survived]
WHAT REMAINS OPEN: [what is unfinished]
WHAT TO WATCH: [the signals that mean they should come back or see a doctor]

Give me:
1. A one-page summary the client can keep and re-read in six months.
2. The three habits they must not drop, stated as actions.
3. A section on what to do if old patterns return - without shame.
4. What I recommend for their next medical review, if anything.
5. A short line on when to come back to me.

Warm, plain, no jargon, no medical claims. Under 350 words.
```

Note *A good discharge summary is a referral engine. Clients re-read it, keep it, and send it to the friend who asks how they did it.*

8 The records retention plan

USE WHEN *Set it up once, before you need it.*

```
Build a records and retention plan for my nutrition practice in India.

MY SETUP: [where records live - phone, laptop, cloud, paper; who has access; whether I have an assistant]

Give me:
1. A simple file structure a solo practitioner will actually maintain.
2. What to keep, where, and in what format, for: intake forms, session notes, food records and photos, consent, payment records.
3. How to handle client photos and food images specifically - they are the most sensitive thing most dietitians hold.
4. What to do when a client asks for their records, or asks to be deleted.
5. What to do with a device that is lost, sold or shared with family.
6. The questions to ask a lawyer or CA about retention periods, rather than guessing.

Do not state legal retention periods as fact. Frame them as questions.
```

Note *Food photos are the most overlooked data in this profession. A client's plate, their kitchen, their family - all of it is personal data.*

9 The AI error correction protocol

USE WHEN *The day you send something wrong.*

An AI assistant produced something I sent to a client, and it was wrong.

WHAT WENT OUT: [the error]
WHO RECEIVED IT: [one client / a group / a public post]
WHAT THE CORRECT INFORMATION IS: [the accurate version]
WHAT THE CLIENT MAY HAVE DONE BECAUSE OF IT: [the risk]

Give me:
1. The correction message to the client - owning it in one sentence, correcting it in one sentence, and giving the right action.
2. The version for a public post, which is shorter and needs no excuse.
3. What to check with the client, given what they may have done.
4. When this must go to a doctor.
5. The one change to my process so it does not repeat.
6. What NOT to say - the explanations that make it worse.

Under 200 words for the client message. No blaming the tool.

Note *'No blaming the tool' is the rule. A client does not care which assistant wrote it; they care that your name is on it.*

10 The scope-of-practice script

USE WHEN *Rehearse it once. You will need it often.*

Write my scripts for staying inside my scope as a dietitian in India, when a client pushes.

Scenarios: [choose the ones that apply - asked to stop a medicine, asked to diagnose from symptoms, asked to read a lab report, asked to prescribe a supplement, asked to advise on a pregnancy complication, asked for advice about a child's treatment]

For each, give me:
1. The line I say, in under 25 words.
2. The bridge that keeps the conversation useful - what I CAN do instead.
3. The referral, named specifically, so it does not sound like a brush-off.
4. What to write in the record afterwards.

Warm, confident, no apology. These must sound like a professional boundary, not a refusal to help.

Note *The bridge is what makes a boundary feel like service. 'I cannot do X, but here is what I can do today' keeps the client.*

PART 9 OF 10

Troubleshooting the Output

Ten fixes for the ten ways AI goes wrong in nutrition work. When the plan sounds American, the numbers look invented, the tone is robotic or the model refuses to commit - this is the page you turn to.

10 PROMPTS IN THIS PART

01 The output is too generic
02 The numbers look invented
03 The food is not Indian
04 The tone is robotic
05 It refuses to commit
06 It gives a different answer each time
07 The two-draft method
08 The plan is too long
09 Building your own prompt library
10 The pre-send review

1 The output is too generic

USE WHEN *When you get a plan that could be for anyone.*

```
The output below is generic. It could have been written for any client in any country.

OUTPUT: [paste it]
MY CLIENT: [the specifics the output ignored]

Rewrite it so that:
1. Every meal names a dish this client's household actually cooks, not a category.
2. The constraints I gave you are visible in the output - the budget, the cooking time, the foods refused, the region.
3. Nothing appears that my client could not buy within a kilometre of their home.
4. Two sentences are removed entirely rather than reworded.

Then tell me which single detail I forgot to give you that would have made the first attempt better.
```

Note *The last question is the real lesson. Nine times out of ten the generic output is a missing-constraint problem, not a model problem.*

2 The numbers look invented

USE WHEN *Every time you see a calorie, gram or percentage.*

```
Review this output for figures I should not trust.

OUTPUT: [paste it]

For every number in the text:
1. List it.
2. Say whether it is a rough estimate, a convention, or something that needs a real source.
3. Say what it would take to verify it properly.
4. Rewrite the sentence so it does not depend on the number at all.

Then give me the final text with every unverifiable figure removed or replaced with a household measure - katori, bowl, glass, chapati, ladle.

Do not replace one invented number with another. If a number is not needed, delete it.
```

Note *Deleting the number is almost always better than correcting it. Plans without numbers are also easier for clients to follow.*

3 The food is not Indian

USE WHEN *Oats, quinoa, salads, 'grilled chicken breast' - the usual suspects.*

This output uses foods my Indian client does not eat, cannot buy, or will not cook.

OUTPUT: [paste it]
MY CLIENT: [region, diet type, budget, cooking setup]

Rewrite it:
1. Replace every foreign or impractical item with the closest Indian equivalent the client already eats.
2. Keep the nutritional role of each item the same - if it was there for protein, the replacement must be a protein.
3. Do not replace with an expensive 'healthy' alternative - replace with what is in a normal kirana.
4. List every swap you made, side by side, so I can check the logic.
5. Flag any swap where the nutritional role is not really equivalent.

Note *Point 5 is the honest part. Soya chunks are not a straight swap for paneer in every client's kitchen, and you should be told so.*

4 The tone is robotic

USE WHEN *When a client message reads like a leaflet.*

Rewrite this so it sounds like a person, not a pamphlet.

TEXT: [paste it]
WHO IT IS FOR: [client relationship, how long I have known them, their language comfort]
HOW I ACTUALLY TALK: [paste two or three messages you have written yourself]

Rules:
1. Keep every fact and instruction. Change nothing clinical.
2. Shorten sentences. Remove every 'it is important to', 'ensure that', 'in order to'.
3. Cut the opening greeting cliché and the closing 'do not hesitate to reach out'.
4. Keep my vocabulary from the samples - if I say 'dabba', keep 'dabba'.
5. Maximum one emoji, or none if my samples have none.
6. Under the same length as the original.

Note *Pasting two or three of your own messages is what makes this work. The model needs your actual voice, not a description of it.*

5 It refuses to commit

USE WHEN *When every answer is 'consult a professional'.*

Your last output hedged on everything and gave me nothing usable.

OUTPUT: [paste it]
WHAT I ACTUALLY NEED: [the decision or draft I am trying to produce]

Redo it with this structure:
1. The general, safe, well-established position - stated plainly.
2. The specific thing I asked for, drafted - clearly labelled as a draft for my professional review.
3. The exact points that genuinely require a doctor, listed separately and briefly.
4. One line at the end: 'This is a draft for a qualified professional to review, not clinical advice.'

Do not add hedging to every sentence. Put the caution in one place, where it belongs.

Note *One consolidated caution is more professional than hedging every line, and it is far more readable for a client.*

6 It gives a different answer each time

USE WHEN *When you need a stable frame, not creativity.*

I need a fixed frame I can reuse so the output stops changing between runs.

TASK: [what you are generating, e.g. a weekly check-in message]
MY CONSTRAINTS: [length, tone, what must always be included, what must never appear]
MY VOICE: [three sample messages of yours]

Give me:
1. A locked prompt I can save and reuse, with placeholders in square brackets only where the content genuinely changes.
2. A list of what I must fill in every time.
3. The three things that were making the output unstable.
4. A one-line version of the prompt for quick use on a phone.

Note *Save the locked prompt as a note on your phone. The best prompt library is the one you can reach in ten seconds.*

7 The two-draft method

USE WHEN *For anything that will reach a client.*

I need to separate my thinking draft from the client-facing draft.

TASK: [what you are working on]
CLIENT: [who it is for]

Give me two versions:
DRAFT 1 - for me only: complete, blunt, including the uncertainties, the things I am not sure about, the alternative I rejected, and what could go wrong. No softening.
DRAFT 2 - for the client: only what is settled, in plain warm language, with the uncertainty expressed as a next step rather than a caveat.

Then list what changed between the two, so I can see what I am choosing not to say and check that the choice is right.

Note *Seeing what you chose not to say is the most useful part. That list is your professional judgement, made visible.*

8 The plan is too long

USE WHEN *When a client will not read it.*

Cut this down to what a client will actually read on a phone.

TEXT: [paste it]
CLIENT: [how engaged they are, how much they read, what they asked for]

Give me:
1. A version under 150 words that keeps the three things that matter most.
2. A version under 60 words for WhatsApp.
3. A one-line version for a story or a status.
4. What you cut, and why - so I can check you did not cut something clinical.
5. What to keep in the longer document, for the client who asks for detail.

Note *Point 4 is the safeguard. You need to see what was removed before you send the short version.*

9 Building your own prompt library

USE WHEN *Month two, once you know what you repeat.*

Help me build a personal prompt library for my nutrition practice.

WHAT I DO EVERY WEEK: [list your recurring tasks - check-ins, plans, handouts, posts, notes]
WHAT I KEEP REWRITING: [the things that feel repetitive]
WHERE I LOSE TIME: [the tasks that take longest]

Give me:
1. A library structure of no more than 12 saved prompts, each named for the job it does.
2. For each: the locked prompt with placeholders, and the one line that makes it work.
3. The three prompts I should build first, ranked by time saved.
4. How to name and store them so I can find one on my phone in seconds.
5. A monthly 15-minute routine to review and improve the library.

No prompt longer than 250 words. These must be usable on a phone between clients.

Note *Twelve prompts, not a hundred. A library you cannot search is a library you do not use.*

10 The pre-send review

USE WHEN *The last step before anything reaches a client.*

Review this draft the way a careful senior colleague would, before I send it to a client.

DRAFT: [paste it]
WHO IT IS FOR: [client profile]
WHAT IT IS: [a plan / a message / a handout / a post]

Check, and report only problems:
1. Any claim about health, weight or outcomes that I cannot defend.
2. Any number that needs a source.
3. Any food or instruction that does not fit this client's stated constraints.
4. Any point where the draft drifts outside a dietitian's scope.
5. Any place the tone would embarrass me - shaming, hype, or over-promising.
6. Anything a client could misread as permission to stop their medication.

If there are no problems in a category, say 'clear'. Do not rewrite the draft - give me the list, and I will fix it.

Note *Make this the last prompt you run on everything. Two minutes here prevents the message you regret at 11 pm.*

PART 10 OF 10

Groups, Challenges and Corporate Work

Group programmes earn more per hour than one-to-one work and reach clients who will never pay for private sessions. These prompts build the structure, the messages and the moderation rules.

6 PROMPTS IN THIS PART

01 The 21-day group challenge

02 Group moderation scripts

03 The corporate four-week programme

04 The school or parent talk

05 The cooking demo plan

06 The group programme debrief

1 The 21-day group challenge

USE WHEN *Your first group programme, or your tenth.*

Design a 21-day nutrition challenge for [audience] in India.

AUDIENCE: [who, how many, what they want, diet types mixed]
MY CAPACITY: [how much time per day I can give this]
PRICE: Rs [amount] per participant

Give me:
1. The promise, in one honest sentence that does not guarantee weight loss.
2. A day-by-day structure in three weeks, one task per day, each doable in under 10 minutes.
3. The three checkpoints where I actually engage, so I am not moderating 21 days.
4. What participants post and where.
5. The two rules that keep the group civil and on-topic.
6. What happens on day 22 - the honest next step, without a hard pitch.
7. The three things that make a challenge fail.

No calorie targets. No before-after photos. No competitive weight-loss leaderboard - explain why that is a bad idea.

Note *Weight-loss leaderboards produce disordered behaviour in group settings. The prompt asks the model to explain why, which also gives you the words for clients.*

2 Group moderation scripts

USE WHEN *Before the group opens, not after the first problem.*

Write my moderation scripts for a WhatsApp nutrition group of [number] people.

GROUP: [who they are, what they paid for, what they expect]

Give me short messages for:
1. The welcome message that sets the rules in under 120 words.
2. When someone gives medical advice to another member.
3. When someone posts a supplement or product link.
4. When someone shares a before-after photo unasked.
5. When someone makes a comment about another member's body.
6. When someone asks a question I cannot answer in the group.
7. When the group goes silent for three days.
8. The closing message at the end of the programme.

Each under 70 words. Firm, warm, no scolding. These must work when pasted in the middle of a busy day.

Note *Have these saved before you launch. The first medical-advice message in a group is where most Indian group programmes get into trouble.*

3 The corporate four-week programme

USE WHEN *The deliverable you sell to HR.*

```
Design a four-week corporate nutrition programme for [company type, size, city].

WHAT THEY ASKED FOR: [their stated need]
WHAT I CAN DELIVER: [my capacity, mode - on-site or online]
BUDGET BAND: [what I am proposing]

Give me:
1. A week-by-week plan: one live session per week, one challenge, one communication piece.
2. What employees get that they would not get from a poster campaign.
3. What the company gets, described in their language, without invented statistics.
4. The materials I need to produce, and which of them can be drafted with AI.
5. How to measure it without collecting personal health data.
6. The report I deliver at the end, and its structure.
7. What to refuse to do, even if they ask.

No claims about productivity, absenteeism or claims data unless I supply a source.
```

Note *Point 5 - measuring without collecting health data - is the ethical answer to the hardest question in corporate wellness.*

4 The school or parent talk

USE WHEN *Where a lot of paediatric and teen clients originate.*

```
Build a 40-minute nutrition talk for [parents of school children / class 10 students / teachers] at a school in India.

SCHOOL CONTEXT: [board, age group, city, what prompted the invite]

Give me:
1. A title that the school will put on a circular.
2. The opening two minutes - one fact, one question to the room.
3. Three sections with one activity each: the tiffin, the exam season, and the packaged snack.
4. The slide outline - maximum 10 slides, one idea each.
5. The five questions parents always ask, with short answers.
6. A one-page takeaway for parents.
7. What NOT to say to a room of parents about their children's bodies.

No calorie targets for children. No BMI discussions in a public setting. No shaming language about weight.
```

Note *Never discuss a child's weight in a public talk. The prompt bans it for a reason - it is the fastest way to create a problem in the room.*

5 The cooking demo plan

USE WHEN *The format that converts best in Indian nutrition.*

Plan a 45-minute healthy cooking demo for [audience] in [city].

MY KITCHEN: [what equipment is available at the venue]
MY SKILL: [how comfortable I am cooking live]
AUDIENCE: [who, how many, diet types]

Give me:
1. Three dishes that demonstrate the three things I most want to teach - chosen for the demo, not for the plan.
2. The prep I must do before, and what to do live so it fits 45 minutes.
3. The talking points while cooking - one per dish, in the client's language.
4. How to handle a dish that fails on the day.
5. The handout with the recipes, in household measures.
6. The closing two minutes and the next step for the audience.

No expensive or hard-to-source ingredients. Everything must be buyable in the local market.

Note *The 'dish fails on the day' script is the difference between a professional and a panicked one. Have it ready.*

6 The group programme debrief

USE WHEN *Day 22, every time.*

Debrief my completed group programme so the next one is better.

THE PROGRAMME: [what it was, how many joined, how many finished]
WHAT HAPPENED: [the high point, the low point, the dropout pattern]
WHAT PARTICIPANTS SAID: [the feedback, including the unkind parts]

Give me:
1. The three things that worked, and how to protect them.
2. The three things that failed, and the specific fix for each.
3. What the dropouts had in common - and whether that is a design problem or a screening problem.
4. The one thing I should stop offering.
5. The revised structure for the next cohort, in one page.
6. The feedback questions I should have asked and did not.

Be direct. Do not soften the failure points.

Note *Whether dropouts are a design or screening problem is the question that separates a hobby from a practice.*

REFERENCE

Appendices A to G

The pages you will actually use more than the prompts: the GLP-1 reality check, the demand numbers, the Indian food reference, the pricing sheet, the pre-send checklist, the 90-day plan and the one-page cheat sheet.

APPENDIX A

The India GLP-1 Reality Check

What changed in 2026, what it costs, and exactly where a dietitian's role begins and ends

Why this appendix exists

Until 2026, weight-loss injections in India were a rich-city curiosity. That changed in a single quarter. Semaglutide lost patent protection in India on 20 March 2026, a wave of Indian generics launched the next day at 50 to 90 per cent below the innovator price, and Novo Nordisk cut the price of Ozempic and Wegovy by up to 48 per cent from 1 April 2026. A therapy that started at roughly Rs 10,850 to Rs 16,400 a month for Wegovy is now available in some form from about Rs 1,290 a month. When a treatment drops from four figures to three, the patient population changes overnight - and every one of those new patients needs exactly the thing a dietitian provides: help eating enough of the right food while appetite disappears.

What this means for your practice

Clients will arrive on these medicines whether or not you raise the subject. Some will be on an innovator brand prescribed by an endocrinologist. Some will be on a generic bought after a newspaper article. Some will be self-medicating. Your role is the same in all three cases: nutrition support, protein and muscle protection, symptom-aware food planning, and a clear line about what belongs to the prescriber. You do not need an opinion on the medicine to do excellent work alongside it.

Monthly cost of GLP-1 therapy in India, as reported in 2026

Brand / maker	Molecule	Reported monthly cost	Notes
Semafull / Semanat (Natco)	Semaglutide (generic)	from about Rs 1,290	Vial format; pen version announced later
Semaglyn / Mashema / Alterme (Zydus)	Semaglutide (generic)	about Rs 2,200 average	Approved for both type 2 diabetes and weight management
Sematrinity (Sun Pharma)	Semaglutide (generic)	Rs 3,000 - 5,200	Indicated for type 2 diabetes
Noveltreat (Sun Pharma)	Semaglutide (generic)	Rs 3,600 - 8,000	Indicated for chronic weight management
Obeda (Dr Reddy's)	Semaglutide (generic)	about Rs 4,200	Pre-filled pen; weight-management approval in process at launch
Ozempic / Wegovy (Novo Nordisk)	Semaglutide (innovator)	from about Rs 5,660	Starting 0.25 mg dose, about Rs 1,415 a week, after the 1 April 2026 cut
Mounjaro / Yurpeak (Eli Lilly)	Tirzepatide	about Rs 13,000	Dual GIP and GLP-1 agonist; the most expensive option in the market

Figures below are from published reporting in March and April 2026, when generic semaglutide entered the Indian market. Prices move. Treat every figure as indicative and confirm current pricing before quoting anything to a client.

The market in numbers, as reported

- India's GLP-1 market crossed roughly Rs 1,400 crore within about a year of the first innovator launches, and CareEdge Ratings has estimated it could grow about fivefold over five years.

- Mounjaro became India's highest-selling drug by value in October 2025, at close to Rs 100 crore in a month, before generics reshaped the market in March 2026.
- More than 50 semaglutide brands were expected to launch in India during 2026, according to drug intelligence platform Pharmarack.
- Analysts quoted at the Ozempic launch expected generic pricing at roughly a 50 per cent discount to the innovator - the actual gap turned out to be far wider.

Yours

Protein-first meal structures for a small appetite. Small-volume, protein-dense Indian foods. Muscle-protection habits alongside resistance training. Symptom-aware eating for nausea, constipation and reflux. The transition off the medicine. Coordination notes to the prescriber. Explaining to a worried family what the plan is for.

Theirs

Whether to start. Which molecule. The dose and any titration. Managing side effects that need medical review. Monitoring bloods. Anything to do with pregnancy, gastroparesis, pancreatitis or a history of eating disorders.

Neither of you

Promising a number on the scale. Endorsing or condemning a brand. Suggesting a client source the medicine themselves. Commenting on whether the prescription was appropriate.

> *One more thing, and it matters. The single most common nutrition failure on these medicines is not overeating - it is undereating protein and losing muscle along with fat. If you take nothing else from this appendix, take that. The client who protects muscle in the losing phase is the client who does not regain everything in the year after.*

APPENDIX B

Why Now: The Demand Numbers

The verified case for specialising, and for using AI in the admin around your practice

The demand signal

Consumer searches for nutritionists and dieticians in India rose 48 per cent year on year, comparing March 2025 to February 2026 with the previous twelve months, according to Justdial data reported in March 2026. The growth was not evenly spread: Delhi recorded a 106 per cent increase and Mumbai 62 per cent. Searches for Pilates rose 74 per cent and personal training 18 per cent in the same period, which tells you what kind of client is arriving - one who is already spending on their health and is looking for the qualified person to complete the picture.

The hiring signal

TeamLease data reported in August 2026 showed openings for nutrition professionals rising 25 to 30 per cent month on month since January, with overall hiring in the segment up nearly 200 per cent between 2024 and 2026. Indeed's data showed nutritionist job postings 26 per cent higher in April 2026 than a year earlier. The GLP-1 wave is part of the reason: hospitals and digital health platforms are building obesity-care teams that need dietitians, not just prescribers.

The supply gap

Against that demand, India has roughly 901 registered dietitians holding the RD credential for a population of about 1.4 billion, according to an IDA workforce audit covering 2021-22. Indian Dietetic Association membership, the closest thing to a national registry in the absence of licensing, is estimated at 10,000 to 15,000 - and that number shrinks when you remove people who have moved into academia, wellness coaching or non-clinical roles. Roughly 46 per cent of the workforce is employed and about 25 per cent is self-employed.

The market size

Grand View Research put India's digital dietitians market at about USD 70.3 million in 2024, projecting about USD 173.4 million by 2030 - a compound annual growth rate of 16.2 per cent. India also carries roughly 101 million people with diabetes, which is the demand base underneath all of this.

What the numbers mean for a solo practitioner

- Demand is growing faster than the qualified supply. Your credential is the scarce thing in this market, not your marketing budget.
- The competition you feel is not other dietitians. It is an estimated 25,000-plus nutritionist accounts on Indian Instagram, many without a registered credential, competing on price and volume.
- More than 60 per cent of qualified nutritionists in India now offer telemedicine, so geography is no longer a limit - which cuts both ways.
- Reported market ranges for Indian nutrition subscription tiers in 2026 run from about Rs 999 to Rs 2,500 a month for a self-guided plan, Rs 2,500 to Rs 5,500 for active coaching, Rs 5,500 to Rs 12,000 for premium intensive work, and Rs 6,500 to Rs 18,000 for medical nutrition therapy with doctor coordination.
- Reported patient lifetime value across six to twenty-four month subscription cycles runs from about Rs 15,000 to Rs 1,80,000 - which is why retention, not acquisition, is the number that decides whether a practice survives.

Where the time goes

A dietitian's week is not mostly counselling. It is intake forms, plan drafting, WhatsApp replies, handouts, follow-up summaries, session notes, content, invoices and the family conversations nobody bills for. That is the work this pack attacks.

Where AI does not help

It does not know your client. It cannot weigh a lab report, a medication list and a family situation the way you do. It will state a wrong figure with complete confidence. Every prompt in this pack is written around that limitation rather than pretending it does not exist.

The position to take

Lead with your credential, compete on outcomes you can actually track, and use AI to remove the writing hours so you can take on the clients the market is already sending you. That is the whole strategy. The rest is execution.

> *A caution on the numbers in this appendix. Market research, search data and reported pricing all move quickly, and different sources measure different things. Every figure here is attributed to the reporting that published it and dated to 2026. Re-check anything you plan to put in a proposal, a pitch or a post.*

APPENDIX C

The Indian Food Quick Reference

What to reach for, region by region, when the model starts writing about oats

How to use this page

Keep it open while you draft. When an output contains something your client's kitchen does not have, replace it from this page rather than letting the model choose again. The point is not authenticity for its own sake - it is that a plan built on food the household already cooks is the only plan that survives week three.

Protein sources by how easy they are to add

- Add to a meal already being cooked: curd or dahi, chaach, paneer, besan in a sabzi, moong dal in a khichdi, sprouted moong, roasted chana, sattu, eggs.
- Add as a side or snack: boiled eggs, chana chaat, peanut chikki (watch the sugar), soya chunks in a curry, rajma or chole in small portions, fish or chicken if the household eats it.
- Needs planning: paneer at home from milk, dal-based cutlets, besan chilla, dhokla, idli with sambar, kheema, machher jhol, curd rice with a dal on the side.

North Indian staples

- Breakfast in under 15 minutes: poha, upma, besan chilla, stuffed paratha with curd, bread with egg, leftover roti with curd and jaggery.
- Everyday protein: dal (toor, masoor, moong, chana), rajma, chole, paneer, curd, chaach.
- The usual traps: double carbs at one meal (roti plus rice plus potato sabzi), a bowl of dal that is mostly water, ghee measured by the spoonful rather than the drizzle, and the evening samosa with chai.

South Indian staples

- Breakfast: idli, dosa, upma, pongal, ragi mudde in Karnataka, puttu in Kerala, appam with stew.
- Everyday protein: sambar with a real dal quantity, rasam, kootu, poriyal with dal, curd rice, fish, eggs, sundal.
- The usual traps: sambar that is thin and mostly tamarind water, coconut in three dishes at one meal, pickle and appalam carrying the salt, and the afternoon filter coffee with sugar on an empty gap.

Bengali and Odia staples

- Everyday: rice, dal, machher jhol, shaak, chorchori, begun bhaja, doi; dalma, saga, santula and chhena in Odisha.
- Protein: fish, egg, dal, chana, curd, and the small fish eaten whole.
- The usual traps: rice blamed for everything when the actual gap is protein at breakfast, mustard oil used generously, and the sweet course treated as a daily right rather than an occasion.

Gujarati and Maharashtrian staples

- Everyday: rotli or bhakri, dal or kadhi, shaak or bhaji, rice, curd, salad; dhokla, khichdi, undhiyu, thalipeeth, usal, misal, pithla, varan bhaat.
- Protein: dal, kadhi with besan, curd, sprouted moong, chana, peanut.

- The usual traps: the farsan and the sweet both being daily rather than occasional, jaggery in the dal and the shaak without being counted, and the ghee on the rotli plus the ghee in the shaak.

Constraints worth knowing by name

- Jain: no onion, garlic, potato or root vegetables. Protein has to come from dal, curd, paneer, sprouts and nuts - plan for it deliberately.
- Satvik and fasting: no onion or garlic; ekadashi and festival fasts often use sama, singhare ka atta, rajgira, makhana, sabudana and fruit.
- No-onion-no-garlic households: common in many communities beyond Jain practice, and usually a religious rather than a taste preference.
- Eggitarian: eggs at breakfast are the single easiest protein fix in an otherwise vegetarian Indian day.

Household measures to use instead of grams

- One katori is roughly the size of a standard tea cup - use it for dal, sabzi, curd and rice.
- One chapati or roti is roughly the size of a small saucer - the client's own palm is the check.
- One glass is about 200 to 250 ml - use it for milk, chaach and water.
- One ladle is roughly two katoris - use it for dal portions served at the table.
- The client's palm, without the fingers, is a reasonable protein-portion reference for paneer, fish or chicken. Their closed fist is a reasonable reference for rice or a sabzi portion.

> *This page is a starting point for language, not a nutrient table. Do not quote any of it as a measured quantity - the whole point is that it is approximate, checkable at a glance, and usable by a client who has never owned a kitchen scale.*

APPENDIX D

The Pricing and Package Sheet

What the Indian market is reported to pay in 2026, and how to work out what your hour is actually worth

Read this first

The ranges below come from market reporting on Indian nutrition practices in 2026. They are indicative, not prescriptive, and they vary enormously by city, credential and specialty. Use them to sanity-check your own pricing, not to set it. Your costs and your capacity decide your floor; the market decides your ceiling.

Reported Indian nutrition subscription tiers, 2026

Tier	Reported monthly range	What it typically includes
Self-guided plan	Rs 999 - 2,500	Customised meal plan, WhatsApp text support, a monthly check-in
Active coaching	Rs 2,500 - 5,500	Weekly call, photo food tracking, daily WhatsApp support
Premium intensive	Rs 5,500 - 12,000	Twice-weekly calls, body composition tracking, lab review
Medical nutrition therapy	Rs 6,500 - 18,000	Doctor coordination, condition-specific work, lab integration

Monthly figures as reported by market analysis of Indian nutrition practices. Confirm against your own city before quoting.

The arithmetic almost nobody does

- Write down your monthly fee. Then write down the total hours that client actually costs you - consultations, plan drafting, WhatsApp replies, the review document, the follow-up you did not bill.
- Divide the fee by those hours. That is your effective hourly rate. Most solo practitioners discover it is between a third and a half of what they thought they were charging.
- Now do it for each tier. The tier with the highest price is very often not the tier with the best hourly rate, because the top tier consumes support time in a way the entry tier does not.
- Fix the tier that is losing money in one of two ways: raise the price, or cap the support. Capping support is usually the honest option, because it forces you to define what the client is actually buying.

What to put in writing, for every tier

- The number of live consultations included, and their length.

- The response window for WhatsApp messages - a real one, that you can keep. Not 'anytime'.
- What is not included, stated plainly. This single section prevents most disputes.
- What happens if the client misses two check-ins in a row.
- The renewal price, written down at the start, so the renewal conversation is not a surprise.

Pricing mistakes that are specific to Indian dietetics

- Pricing per session rather than per programme, which makes every follow-up a fresh purchase decision for the client.
- Discounting in the first message. It signals that the first price was not real.
- Competing with a Rs 499 online plan on price. You cannot win that, and the client who buys on price leaves on price.
- Not charging for the documents. The plan, the handout, the review summary and the discharge note are hours of work. They belong inside a package price, not outside it.
- Quoting a monthly figure without ever saying the per-day equivalent. Rs 3,000 a month is Rs 100 a day, and saying so is not a trick - it is a clearer description of the same number.

Where AI genuinely saves money in a practice

- Drafting time on plans, handouts and review documents - the largest single block of unpaid hours in a solo practice.
- Content production, which otherwise competes directly with billable client time.
- The admin around enquiries, which is where leads are lost while you are in a consultation.
- Note-writing, which is the task most practitioners do at 11 pm and resent most.

> *One rule about money and this pack. Never let an AI tool invent a price, a discount, a refund policy or a tax figure for your practice. It does not know your costs and it does not know current Indian tax law. Paste your real numbers in, and keep the tax questions for your chartered accountant.*

APPENDIX E

The Pre-Send Review Checklist

Twenty checks. Two minutes. Run before anything reaches a client

Why a checklist and not a feeling

AI output reads fluently, and fluency feels like accuracy. That is exactly the trap. A checklist forces you to look at the specific places where confident-sounding text goes wrong in nutrition work. Use it on every plan, every handout, every message that goes to a client, a group or a public page.

- ■ No diagnosis anywhere, including in a caption or a group reply.
- ■ No medication advice, no dose, no 'you could reduce' anything.
- ■ No calorie, gram or macro figure that I did not set myself.
- ■ No statistic, study or percentage that I cannot point to a source for.
- ■ No claim of weight loss, reversal, cure or a specific outcome.
- ■ No food described as bad, junk, forbidden or a cheat.
- ■ No body-shaming language, including in the client's own words quoted back.
- ■ Every dish is something this household actually cooks.
- ■ Every ingredient is buyable within a kilometre of where this client lives.
- ■ The client's stated refusals and allergies are respected - checked line by line.
- ■ The budget constraint is visible in the output, not just in my prompt.
- ■ The cooking time constraint is realistic for this client's weekday.
- ■ Festival, fasting and family constraints are handled, not ignored.
- ■ The medical caveat appears once, clearly, at the end.
- ■ Anything requiring a doctor is flagged as such, specifically, not vaguely.
- ■ The tone sounds like me, not like a leaflet.
- ■ The length is what this client will actually read on a phone.
- ■ No client name, phone number, address or report identifier appears anywhere.
- ■ The document does not read as permission to stop any medication.
- ■ If this ended up on social media tomorrow, I would still defend every line.

> *If you cannot tick all twenty, do not send it. Rewrite the line that failed, or cut it. A shorter document that is right beats a complete document that is wrong.*

APPENDIX F

The 90-Day Implementation Plan

One action a week. Do not do this all at once

How to run this

The most common failure with a pack like this is using thirty prompts once and none of them twice. Pick one prompt a week, use it on real clients, and keep it only if it saved you time. By week twelve you will have a personal library of about a dozen prompts that do 80 per cent of your writing. That is the goal - not using everything.

Weeks 1 to 4 - the foundation

- Week 1: paste the Master Context block, then run the Master Client Brief (Prompt 1) on your next two new clients. Compare the output with what you would have written yourself.
- Week 2: run the red-flag triage (Prompt 7) on every new enquiry for a week. Note how many you would have taken without checking.
- Week 3: build your WhatsApp intake form (Prompt 2) and use it on all new enquiries. Measure how many reply in full.
- Week 4: run the Master Meal-Plan Frame (Prompt 13) for one client per diet type you see most. Edit heavily the first time; the edits are your method, written down.

Weeks 5 to 8 - the volume work

- Week 5: set up the photo food-log protocol (Prompt 43) with your current clients. Track how many actually send photos by day seven.
- Week 6: write three handouts from Part 5 that you repeat most often. Save them as your own templates.
- Week 7: if you have any GLP-1 clients, run Prompts 29 to 31 and build your scope script from Prompt 104. If you do not, build the prediabetes 12-week frame (Prompt 34) instead.
- Week 8: build the two-draft method (Prompt 111) into your routine and run the pre-send review (Prompt 114) on everything for one week.

Weeks 9 to 12 - the business

- Week 9: run the positioning prompt (Prompt 83) and rewrite your bio from the result.
- Week 10: rebuild your packages using the four-tier prompt (Prompt 84) and work out your effective hourly rate per tier. This is usually the week that changes the year.
- Week 11: build the 30-day content calendar (Prompt 72) and record the three reels it recommends first.
- Week 12: run the monthly revenue review (Prompt 92) and the group programme debrief if you ran one. Then prune your prompt library to the twelve you actually use.

What to measure

- Hours saved per week on writing. Track it for the first four weeks, then stop - you will know.
- Reply time to new enquiries. This is the number most closely tied to conversion.
- Client retention at week six and week twelve. If it moves, the follow-up prompts are working.
- Your effective hourly rate. Everything else is a proxy for this.

> *Ninety days is enough to know whether this belongs in your practice. If a prompt has not earned its place by then, delete it. A working library of twelve beats an unused library of a hundred and twenty.*

APPENDIX G

The One-Page Cheat Sheet

Print this. Stick it next to your laptop

The ten rules

1. AI drafts, you decide - nothing goes out unread. 2. No numbers you did not set. Ask for dishes, not grams. 3. No diagnosis, ever, in any format. 4. One caution at the end, not a hedge in every line. 5. Strip the identity before you paste - age band, sex, city is enough. 6. Verify every external fact, especially prices and drug brands. 7. The client's own food wins - dal, roti, rice, sabzi, idli, poha, curd. 8. A plan the family will not cook is a plan that has already failed. 9. The follow-up is the product. The plan is 20 per cent of it. 10. If it ended up public tomorrow, you should still defend every line.

The five prompts to memorise

The Master Client Brief (1). The Master Meal-Plan Frame (13). The GLP-1 intake add-on (29). The 'I cheated' reply (46). The pre-send review (114). Those five cover the moments that decide whether a client stays. Everything else in this pack is an optimisation on top of them.

The one sentence that keeps you out of trouble

When a client asks something that belongs to a doctor, the answer is not 'I cannot help' - it is 'that one is your doctor's call, and here is what I can do today'. Say the second half. Always say the second half.

> *This pack is a tool for a qualified professional. It is not clinical guidance, it is not a substitute for your training, and it does not know your client. Every prompt in it assumes a dietitian is reading the output before a client does.*

A final note

The dietitians who will do well in India over the next few years are not the ones with the best prompts. They are the ones with a credential they can defend, outcomes they can track, and enough time in the day to actually speak to their clients. Everything in this pack exists to buy back the third one.

Use it on real clients this week. Keep the twelve prompts that earn their place, delete the rest, and rebuild the library around the way you actually work. A working tool that fits your practice beats a comprehensive one that sits in a folder.

Faisal Fayaz Lone
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